

18 May 2023

Lucy Letby - Cross-examination Day 2: Babies A, B & C

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

Thursday, 18 May 2023

(10.30 am)

(In the presence of the jury)

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

MR JOHNSON: Ms Letby, yesterday you answered several questions from me on several issues. Is there anything that you said during the course of my questioning to you that you have thought about overnight and you want to modify any of the answers that you have given?

LETBY: No.

JOHNSON: One of the statements of fact that you made to the jury was that student nurses are not given handover sheets.

LETBY: That's right.

JOHNSON: Was that true?

LETBY: Yes.

JOHNSON: When you were a student nurse, were you given any handover sheets?

LETBY: Sometimes, yes, but the neonatal unit didn't tend to give them out to students at that point.

JOHNSON: Could just repeat that answer because I'm not quite sure I understood it.

LETBY: What do you mean? As a student nurse, yes, I had handover sheets from certain areas, but routinely the neonatal unit at the Countess at that time did not provide students with a handover.

JOHNSON: I just want to understand this answer: when you were a student at the Countess of Chester Hospital, were you given handover sheets?

LETBY: At some placements, yes.

JOHNSON: Some placements? Do you mean in different parts of the hospital?

LETBY: Yes.

JOHNSON: Well, all I'm interested in is the neonatal unit, so if you would concentrate on that. Were you given handover sheets from the neonatal unit?

LETBY: I can't recall specifically.

JOHNSON: You know where I'm going, don't you?

(Pause)

MR JUSTICE GOSS: Do you know where Mr Johnson's going? No?

She shook her head. She doesn't know where you're going.

MR JOHNSON: Did you have any handover sheets at your house from when you were a student at the Countess of Chester Hospital?

LETBY: I couldn't say. I've got handover sheets, I don't know where they're from.

JOHNSON: Is that true?

LETBY: Yes.

JOHNSON: What was the first day that you worked at the Countess of Chester Hospital in the neonatal unit?

LETBY: 2 January 2012.

JOHNSON: As a student?

LETBY: When I was a student? I don't recall the date. It would have been between 2008 and 2011, I can't recall specifically what dates all my placements were.

JOHNSON: I'm going to suggest to you that the first day you worked on the neonatal unit as a student was 1 June 2010.

LETBY: Okay.

JOHNSON: And the reason I'm making that suggestion to you is that there is a handover sheet in your house -- or was a handover sheet in your house from that date?

LETBY: Yes.

JOHNSON: Are you agreeing with me?

LETBY: Yes. At the time that we're talking about, students did not have handover sheets.

JOHNSON: Why didn't you say that when I asked you the question a few minutes ago, that we did get handover sheets when I was a student but the students no longer get handover sheets?

LETBY: I believe I did say at the time that the practice now is that they do not have a handover sheet.

JOHNSON: Are you doing your best to tell the truth, the whole truth and nothing but the truth, Lucy Letby?

LETBY: Yes.

JOHNSON: Where was that handover sheet dated 1 June 2010?

LETBY: I have no idea.

JOHNSON: Did you have a keepsake box with roses on it in your house at 41 Westbourne Road?

LETBY: Yes.

JOHNSON: What was in the keepsake box?

LETBY: I can't recall from memory.

JOHNSON: One of the things in the keepsake box was that handover sheet, wasn't it?

LETBY: I don't know. I don't have any recollection of where the handover sheets were.

JOHNSON: I'm going to show it to you as the police recovered it, please. It's exhibit WG3.

Just have a look at that, if you would. Is that from your house?

LETBY: I can't say. I don't recall this specific individual handover sheet, no.

JOHNSON: Just take it in your hands. What's unusual about it?

Pick it up. What's unusual about it?

LETBY: I don't know what you mean. It's a standard handover sheet.

JOHNSON: It is. But how does it differ from all the other handover sheets that you took home?

LETBY: It hasn't got handwriting on it.

JOHNSON: That's one difference. Another is it hasn't got any folds in it either, has it? It's in pristine condition.

LETBY: Is this the original?

JOHNSON: It is the original. In the keepsake box in your house.

LETBY: Okay.

MR JUSTICE GOSS: Well, let's just be clear. Do you agree it hasn't got any folds in it?

LETBY: I agree, yes.

MR JUSTICE GOSS: All right, thank you.

MR JOHNSON: You knew where you were keeping these handover sheets, didn't you?

LETBY: No. They were in several locations, I believe, throughout the property.

JOHNSON: They were. And there were 99 handover sheets from your time as a student, weren't there, in your house?

LETBY: I can't comment on that, I don't know how many there were.

JOHNSON: Does that sound about right?

LETBY: I wouldn't know. I don't know how many handover sheets I had in different locations until I've been told now how many there were in total.

JOHNSON: You are not, and you have not been prepared to tell the truth about these handover sheets, have you?

LETBY: The truth is what I've told you.

MR JOHNSON: May I formally exhibit that handover sheet, please, my Lord?

MR JUSTICE GOSS: Yes.

MR JOHNSON: I want to deal with a bit of evidence that you gave yesterday on the subject of handover sheets. If Mr Murphy would help me, please, by opening up on the screens tile 331 in the [Baby O] sequence, please.

Do you remember giving us some evidence about this yesterday?

LETBY: Yes.

JOHNSON: And your explanation, I'm going to suggest, and I'd ask you either to agree or disagree, was that you took the handover sheet for 23 June home and the reason that you took it home was because on the back of it you had written down details of drugs that had been given to [Baby O] or another child that you were looking after that day. Am I right so far?

LETBY: There was documentation on there. I'm not sure exactly what was on there, but yes, the handover sheet did have writing on it, yes.

JOHNSON: Well, I'll remind you of exactly what you said, seeing as we don't seem to be going in the same direction. You said to Mr Myers:

"It's common practice that we will utilise the handover sheets. So we would write on the back of the handover sheet and if something significant happens, we would just grab the nearest available paper or paper towel, something to write on so that we can then document retrospectively."

Then Mr Myers asked you this question:

"When in this note [and he's referring at this point, Mr Myers, to what we can see on the screen] you finish, 'Left off signing for drugs until tomorrow', can you explain to us what that refers to?"

And you said:

"So I hadn't completed all of the documentation for the medications."

So "for the medications", concentrate on those words, please:

"And as I was back in work the next day, I planned to do it then."

You were then asked this question:

"What was happening with whatever notes you had in between the end of your shift and completing writing up medications the next day?"

So do you agree with me that the essence of the questions and answers is that this is all to do with medications?

LETBY: Medication was included, yes. I don't know that it's medications in its entirety.

JOHNSON: And you said:

"I needed it for the next day."

You were then asked the question:

"So where was it?"

Talking about the note. And you said:

"It came back."

In other words, you took it back home. That's what you meant, wasn't it?

LETBY: I took it to bring it back into work the next day, yes.

JOHNSON: Yes:

"Where was it?"

Said Mr Myers. And you said:

"At this point in my pocket."

And you confirmed that the notes were in your pocket.

Then there was a bit of confusion between the two of you and Mr Myers said:

"Probably my fault. You've got the notes in your pocket. You've said something about 'left signing up drugs until tomorrow', which we can see on the screen. You're talking about walking home. What's going to happen to those bits of paper?"

And you then said:

"They're going to come back to work the next day."

By that, you meant you were going to take them into work the next day, didn't you?

LETBY: Yes.

JOHNSON: Yes. And Mr Myers said:

"Question: 23 June, you have the handover sheet, which includes on it [Baby P] and [Baby O] --

"Answer: Yes.

"Question: -- and the various notes on the back of that?

"Answer: Yes.

"Question: Is that the handover sheet that you would have taken home with you at the end of the shift, if you remember?

"Answer: Yes. So that's the exchange between you. There, you were saying that you needed -- you took the handover sheet home deliberately; do you accept that?

LETBY: Yes.

JOHNSON: The reason for taking it home deliberately was because you needed it for the next day?

LETBY: Yes.

JOHNSON: And the reason you needed it for the next day was because you hadn't completed writing up the medications and you needed the notes on the back of the handover sheet to remind you of what you needed to write the next day; do you agree with that?

LETBY: Or for any other documentation that still needed to be completed.

JOHNSON: No, no. The question and the answers were very specific: it was all to do with medications, wasn't it?

LETBY: Well, here, yes, I'm referring to drugs, yes.

JOHNSON: Yes. And the question and answers between you and Mr Myers were referring to drugs, weren't they?

LETBY: Yes, if that's what it said, yes.

JOHNSON: Right. I'm going to show you the handover sheet, the original handover sheet. Just for the record, what I'm about to hand over is exhibit 15, which has already been exhibited in the trial. It is, just in case these documents get separated, four

handover sheets that were in the Ibiza bag: 23 June, 24 June, 25 June and 28 June.

So they're all on a paper clip at the moment with a court sticker on the front sheet.

(Handed)

What I'd like to do also is give the jury photocopies. I'd invite people to put this behind divider 10 in jury bundle 1.

I'm going to ask Mr Stansfield to give you a copy of what the jury have, which is a redacted or edited copy.

I'm going to ask you to confirm, if you would -- could you take the original, the first sheet of the originals out from the -- that's it. Would you compare, and take as much time as you like, the original to the copy that we've just given to the jury, a version of which you have as well? And would you confirm that it is the same document, apart from the fact that the names of the children other than "Male [Surname of Baby Q]", who we know is [Baby Q], and [Baby P] and [Baby O], have all been edited out?

LETBY: Yes.

JOHNSON: Would you now turn over the original and compare it to the second page that I have just handed you as a copy?

LETBY: Yes, it's the same.

JOHNSON: The same document. Right. I'm not interested in the front of this document for these purposes. I'm interested in the back and your notes. You point out to us where the notes of the medications are you have made on the back of that document.

LETBY: There's a note about caffeine and [Baby P].

JOHNSON: Right. So this is the top left; yes?

LETBY: Yes.

JOHNSON: "Caffeine" underlined?

LETBY: Yes.

JOHNSON: Right, okay. Anything else for [Baby P]?

LETBY: No.

JOHNSON: [Baby O]?

LETBY: No, the drugs were written on a paper towel, I believe.

JOHNSON: The child whose name has been edited out, who was the third child in nursery 2?

LETBY: There's no medications.

JOHNSON: So your evidence to the jury yesterday is that you took that home because it had caffeine written on it and you brought it in the next day to help you remember, what caffeine, is that the thing you're struggling --

LETBY: I didn't say that, I didn't say I'd taken it because of caffeine.

JOHNSON: No, you said you took it for the drugs.

LETBY: Yes, with the paper towel that was also with it.

JOHNSON: There's a paper towel with it?

LETBY: Yes, we've seen the paper towel, haven't we?

JOHNSON: Well, there was a -- there have been a couple of paper towels. I'm interested -- you were answering questions about the handover sheet. That's why I started with reminding you of what your evidence was. Were you telling the truth to the jury yesterday?

LETBY: Yes.

JOHNSON: The Morrisons bag. When did that bag go under your bed?

LETBY: I can't recall a specific date when I would have put that under the bed.

JOHNSON: Well, give us a range of dates if you would, please.

LETBY: Well, the Ibiza bag sort of replaced that bag, so it would be after -- just before that time maybe or after.

JOHNSON: So just before -- so some time around the 20-something-th of June 2016?

LETBY: I can't be specific, but the Ibiza bag became my new work bag, yes.

JOHNSON: Right. And in the Morrisons bag there were 31 handover sheets, we've heard, haven't we?

LETBY: Yes.

JOHNSON: And they were dated between July 2014 and then April 2015 through to June 2016; do you accept that?

LETBY: Yes. Obviously, I can't see it now but if that's been agreed, then yes.

JOHNSON: I can produce the sheets. Would you like to see them?

LETBY: No, if it's been agreed...

JOHNSON: How did all those handover sheets get into that bag?

They didn't get there on their own, did they?

LETBY: No.

JOHNSON: So you tell us the process between emptying out your uniform when you're about to launder it to those handover sheets ending up in that bag, please.

LETBY: When I've come home from work and I've taken my uniform off, the contents of the pocket have gone into the work bag.

JOHNSON: And where does the work bag then go on the next day, the next work day?

LETBY: To work.

JOHNSON: Yes. So, what, you're ferrying handover sheets to and from the Countess of Chester, is that what you're saying?

LETBY: I can't say definitively.

JOHNSON: Well, that must be what you're saying, mustn't it?

LETBY: I accept that they're all in that bag, yes.

JOHNSON: Well, they're all in the bag when the police find it.

LETBY: Right.

JOHNSON: What I'm more interested in is, first of all, why you put them into that bag at all.

LETBY: I can't recall.

JOHNSON: Can't or won't?

LETBY: No, I collect -- paper just gathers. They were just bits of paper to me.

JOHNSON: You're not telling the truth, are you?

LETBY: I am.

JOHNSON: Why don't you want to tell the truth?

LETBY: That is the truth. They have no meaning to me at all, they're just pieces of paper.

JOHNSON: If they have no meaning to you, why do you keep them?

LETBY: Because I keep a lot of paper, I have difficulty throwing anything away. I have copious amounts of paper, cards, notes throughout my whole life. These are no different: they're just pieces of paper that have been gathered and I've never done anything with them.

JOHNSON: Well, you have: you've put them into different bags in different places.

LETBY: Yes, as a piece of paper, not because of the content.

I've moved paper round my whole life.

JOHNSON: Oh yes, you have, and that's one of the reasons I asked you the questions about where you were living at various points.

LETBY: Yes, I accept possessions came with me to multiple addresses, and that's including these handover sheets as well as many other handwritten things: cards, letters, notes. It's the paper that I accumulate, not the content.

JOHNSON: The question that the jury may be interested in is: why?

LETBY: I have difficulty throwing anything away.

JOHNSON: Is that why you bought a shredder?

LETBY: I did have a shredder at some point, yes.

JOHNSON: Yes. That wasn't the question. The question is: is your difficulty in throwing things away the reason why you bought a shredder?

LETBY: I bought a shredder for certain documents once I bought the house, yes.

JOHNSON: Right. Which documents were going to go in the shredder?

LETBY: Predominantly bank statements.

JOHNSON: Why not handover sheets?

LETBY: I didn't know I had the handover sheets, they were insignificant to me at that time.

JOHNSON: They were in your work bag which was going to and from work every day.

LETBY: With lots of other paper as well.

JOHNSON: Where's all that other paper gone then?

LETBY: I have various amounts of paper at home of different things --

JOHNSON: No, no, don't deflect the question.

LETBY: I don't know what you mean, sorry.

JOHNSON: We're talking about the work bag. Where is all the other paper in the work bag?

LETBY: I'm talking about there's multiple handover sheets in there.

JOHNSON: Yes. So they're going to and from work with you every day. Every day when you empty out your uniform, you add to the pile; is that right?

LETBY: I must have done. I can't recall specifically every day what action I took.

JOHNSON: Well, of course not, on an individual basis, but what were you thinking as this pile of handover sheets accumulated to almost the size of a phone book?

LETBY: They weren't the size of a phone book in one area, I didn't recall that I had that many in different locations.

JOHNSON: Well, don't argue about whether it's a phone book or not. What were you thinking as you were accumulating the handover sheets?

LETBY: I wasn't thinking anything, they're just bits of paper.

JOHNSON: So put them in the shredder.

LETBY: I didn't shred anything particularly other than bank statements.

JOHNSON: When did you buy the shredder?

LETBY: I can't recall an exact date that I bought a shredder.

JOHNSON: I'm not asking for the exact date. Did you have it in Ash House?

LETBY: No.

JOHNSON: So you bought it some time between 5 April 2016 and the date that the police knocked on your door?

LETBY: Yes.

JOHNSON: Why didn't you shred the handover sheets?

LETBY: Because they're insignificant. I haven't shredded a lot of documents in my house that could have been shredded.

JOHNSON: These are very significant documents, aren't they, Lucy Letby?

LETBY: Well, they are sitting here now. They weren't to me at that time. They're just pieces of paper.

JOHNSON: They have the names on dead children on them, don't they?

LETBY: They have the names of a lot of children on them, yes, and they shouldn't have come home, I agree, but they were in a home with me where nobody else was, they were confidential, they were just put in amongst a lot of other notes and bits of paper, study notes that I've kept my whole life.

JOHNSON: Are you really asking the jury to accept that pieces of paper with sensitive information about dead children are insignificant?

LETBY: Yes.

JOHNSON: I'd like to look at [Baby M]'s blood gas document, please, which is in his sequence of events at tile 171.

It's also J18905 -- I've got the wrong reference.

Let me give you the original. I've got the wrong reference for the point I want to make. This, my Lord, was produced to the court on 21 February this year.

It's exhibits 7 and 8.

These documents were in your Morrisons bag, weren't they?

LETBY: Yes.

JOHNSON: Are they insignificant?

LETBY: At the time of being found, yes.

JOHNSON: So a child who brushed with death, these are his blood gas readings taken whilst he was being resuscitated, you regard those as insignificant?

LETBY: I think the context is different. These have come home with me, with many babies, they're not just for babies on the indictment. It's not insensitive, they've just come back as pieces of paper and have ended up just being treated that way, as bits of paper.

JOHNSON: They haven't come home with you, have they? You took them home.

LETBY: I took them home, yes, but not with any intention.

JOHNSON: Why do you keep saying "came home with me" as if they were active, as if they were doing the action?

LETBY: I apologise if -- that's my turn of phrase, but that's how I see it. Came, took, whichever, they've come back with me, I took them back, however you want to word it.

JOHNSON: It's you trying to separate yourself from a conscious act, isn't it?

LETBY: No.

JOHNSON: Do you remember what you said about these documents to your counsel a couple of days ago?

LETBY: About this specific document?

JOHNSON: Yes.

LETBY: No, I don't recall that it was presented to me during that time.

JOHNSON: You were asked about it.

LETBY: Right.

JOHNSON: What you said to -- the gas print, okay, we'll deal with that first of all. Open it out to remind yourself of what it says:

"Question: Did you use the machine?"

You answered the question with a question:

"Answer: At this time?"

Mr Myers followed up with:

"Question: During the course of what happened with [Baby M].

"Answer: Yes."

Was that true?

LETBY: Did I use the blood gas machine for [Baby M]? Is that what you're asking?

JOHNSON: That was the question he was asking you.

LETBY: Well, I can't possibly say for definite. This is a printout, yes.

JOHNSON: "Why would you have that item at home, can you help us with that?"

You were asked. And as you do, you said:

"Answer: It's come back with me from work in my uniform.

"Question: Once you've got the printout from the machine, what would you have done with it in real time on the unit?

"Answer: I would have put it in my pocket and taken it to the cot side to then write up on the charts that we've seen, the handwritten charts.

"Question: The question that arises is why it doesn't just go in the bin after that. Why doesn't it?

"Answer: That is an error on my part."

Yes?

LETBY: Yes.

JOHNSON: Was that true?

LETBY: Yes.

JOHNSON: Do you remember a nurse called [Nurse B]?

LETBY: Yes.

JOHNSON: You know her very well, don't you?

LETBY: I know her as a colleague, yes.

JOHNSON: Do you remember what [Nurse B]'s evidence was about that printout?

LETBY: No.

JOHNSON: Really?

LETBY: I can't recall that, no. We've heard a lot of information. I can't recall that, sorry.

JOHNSON: Well, let's look at tile 171, please, Mr Murphy.

What's the time on the document you've got in front of you?

LETBY: 16.22.

JOHNSON: Do we see the results recorded on the blood gas chart?

LETBY: Yes.

JOHNSON: Whose handwriting is that?

LETBY: [Nurse B]'s.

JOHNSON: Do you now remember what [Nurse B] said about that blood gas reading?

LETBY: No.

JOHNSON: She said that she took it, she wrote it on to the chart.

Do you remember what she said about what she habitually does with the printout?

LETBY: No.

JOHNSON: What do you think your colleague [Nurse B] --

LETBY: She disposed of it.

JOHNSON: Yes. Where would she dispose of it, do you think?

LETBY: In the confidential waste bin.

JOHNSON: Yes. When did you fish it out of the confidential waste bin and take it home?

LETBY: I've never fished anything out of the confidential waste bin.

JOHNSON: How did you get it?

LETBY: I can't recall specifically.

JOHNSON: It was for your little collection, wasn't it, Lucy Letby?

LETBY: No.

JOHNSON: They could have gone in the shredder, couldn't they?

LETBY: None of the paper that I had went into the shredder.

JOHNSON: No, no, that wasn't the question: they could have gone into the shredder, couldn't they?

LETBY: Could have, yes.

JOHNSON: How much is a shredder?

LETBY: I -- I don't think that's fair to ask. I don't know.

JOHNSON: You bought one.

LETBY: I can't recall a specific date and time when I went into a shop and bought a shredder and how much that cost.

JOHNSON: Did you have money to burn?

LETBY: What do you mean, sorry?

JOHNSON: Did you have so much money that you could just buy whatever you want whenever you want it?

LETBY: I don't -- I don't understand what finance has...

JOHNSON: Well, the jury may be interested, and I'm certainly interested, in why you'd buy an electrical appliance that you have no real intention of ever using.

LETBY: Well, I did use it for bank statements.

JOHNSON: Yes. Why did you lie about the shredder in interview?

LETBY: I couldn't recall at that time whether I did have a shredder or not. A shredder is an insignificant item to have in your home.

JOHNSON: It's like the pieces of paper?

LETBY: Yes.

JOHNSON: It's insignificant?

LETBY: Yes.

JOHNSON: Do you remember the explanation you gave for what I suggest is -- well, let's go to the interview just to remind ourselves of what you actually said. It's in the overarching interviews, which are in the second of the lever arch files for the interviews, please. It's going towards the back and it's behind divider 2 -- sorry, behind divider 3. So OA3. It's about two-thirds of the way down the page, so I'll just show you where I'm looking, page OA39.

Do you see, just above the second of the rings of the ring binder of the lever arch file, there is a time that says 00.08.41?

LETBY: Yes.

JOHNSON: And you actually brought up the shredder as we look at the questions and answers. So if we go up about a couple of inches from there, you're asked the question -- just above halfway down the page, they're asking about the sheets and you say:

"Question: Do these sheets that are in your folder that you've kept at your home address, Lucy, relate to babies which you were designated nurse for?

"Answer: Yes, they're all babies that are on the unit at that point whether you look after them or not, so yeah.

"Question: Okay. Have you previously taken any of these handover sheets home and disposed of them?"

And what's your answer to that question?

LETBY: "No I don't think so because I haven't got a shredder and that's how I would -- that's how I would have to get rid of them".

JOHNSON: So it's not the police suggesting a shredder to you, is it?

LETBY: They're suggesting ways in which I would have disposed of those handover sheets at home.

JOHNSON: Why do they suggest a way of getting rid of?

LETBY: They've asked me have I ever taken the handover sheets home and disposed of them.

JOHNSON: Yes. How is that a suggestion of them -- the means by which you'd get rid of them?

LETBY: That's how I took disposal to mean, a confidential disposal.

JOHNSON: They never suggest shredder to you, do they? They don't say, "Have you got a shredder"?

LETBY: No.

JOHNSON: To which you mistakenly answer no. They say, "Why haven't you got rid of them", and you say, "Because I haven't got a shredder".

LETBY: I couldn't recall at that time whether I did have a shredder or not. I'd just been arrested by the police. Locating a shredder wasn't on my mind.

JOHNSON: Well, the shredder was on your mind because you were the person that introduced it into the interview.

LETBY: Yes, in reference to the word disposal.

JOHNSON: Are you telling the truth, the whole truth and nothing but the truth?

LETBY: Yes, I am.

JOHNSON: Do you remember the explanation you gave to the jury on 2 May about that question and answer?

LETBY: No, I can't recall all of the evidence that I've given.

JOHNSON: What you said then, it's page 163 of the transcript if anyone wants to check, is:

"It was an oversight. I had forgotten that I did have one. I only acquired it fairly recently."

What did you mean by that?

LETBY: That it was an oversight for me to say that I didn't have a shredder at that time and that I had bought one recently in relation to the date of interview.

JOHNSON: So help us again: when did you buy the shredder now that you've been reminded of that that you remembered about 2 weeks ago?

LETBY: Shortly before this interview.

JOHNSON: I'm sorry, I think your answer got lost by overtalking.

LETBY: What was the question, sorry?

JOHNSON: When did you buy the shredder now that you've been reminded of the fact that you remembered it in interview?

LETBY: Shortly before this interview if I've said that it was bought recently.

JOHNSON: There was a shredder box at your parents' home in Hereford, wasn't there?

LETBY: Yes.

JOHNSON: And that was in your room, as I understand your evidence?

LETBY: Yes.

JOHNSON: Did that box ever leave your parents' house as far as you know?

LETBY: I can't recall specific locations of many boxes over the years, no. But it probably did move with me, yes.

JOHNSON: Moved with you? Well, this is a box for your parents' shredder as I understood your evidence. Did I understand it correctly?

LETBY: The box in the wardrobe?

JOHNSON: Correct. Do you want me to show you a picture just to remind you?

LETBY: No, I'm okay. So yes, I believe that was my parents' shredder, yes.

JOHNSON: Is there some doubt in your mind?

LETBY: I can't recall definitively all the shredders that were in the property and whose belonged to who.

JOHNSON: Did the Letby family have several shredders?

LETBY: No.

JOHNSON: "All the shredders that were in the property and who they belonged to", what did you mean by that?

LETBY: I mean recalling exactly where a shredder is or the box that you've kept it in or where it was bought isn't something that would be a date ingrained on my mind.

I don't know where it came from or when it moved, if it moved.

JOHNSON: Just remind us because I may have misunderstood what you said on 2 May. If I did, I apologise. But why did you write -- if we could have the picture, actually, because it would help, I think, Mr Murphy, if you wouldn't mind.

It's image 47, please.

I think ultimately you settled on the fact that it was you who wrote "keep" on the box; is that right?

LETBY: It may well be.

JOHNSON: It may well be or it is or --

LETBY: I can't definitively say that is my writing, but it does look like it, yes.

JOHNSON: Just remind us why you wrote "keep" on that box?

LETBY: To keep the box and the shredder.

JOHNSON: Well, what you said, because his Lordship actually asked you questions about this because I think we were all a little unclear on what you were saying on 2 May. What you said in answer to questions from his Lordship was this was an instruction to your parents to keep the shredder. Did I misunderstand that?

LETBY: No, so the box is labelled in relation to the shredder.

JOHNSON: But there's no shredder in the box?

LETBY: No, but it's in the house.

JOHNSON: Why do you need to write "keep" on something that's in your room in your parents' house?

LETBY: I can't answer that.

JOHNSON: Do you remember an answer you gave yesterday about your parents going into your room?

LETBY: Yes.

JOHNSON: They never go in your room. That's what you said.

LETBY: Yes. They wouldn't go through my things, no.

JOHNSON: So the question I would like you to answer is: if that is true, why do you write "keep" on something as an instruction to your parents when that something is in your room that they never go into?

LETBY: I can't answer that.

JOHNSON: Is the truth of it that you are making up bits of evidence as you're going along?

LETBY: No.

JOHNSON: Because we know, don't we, what was in the shredder box?

LETBY: Now I've been told, yes.

JOHNSON: Handover sheets.

LETBY: Yes.

JOHNSON: And that's why you wrote "keep" on the box, wasn't it?

LETBY: No.

JOHNSON: Can we go to the -- actually, can I just ask you -- it's my fault, I'm allowing you to accumulate things in front of you, which probably isn't very helpful. So if I could retrieve from you, please, with the assistance of Mr Stansfield, at least the exhibits, otherwise I'll get into trouble for mixing things up.

(Pause)

MR JUSTICE GOSS: I think the photocopy of the handover sheet --

MR JOHNSON: That doesn't need --

MR JUSTICE GOSS: That should go into the jury bundle in the witness box, section 10.

MR JOHNSON: We'll sort that out in the break, my Lord.

I'll just hand all these back.

I want to deal with the card you sent to the [Family of Baby I] next. That's in the fourth sequence for [Baby I] at tile 296, please.

This photograph was taken by you on the day of [Baby I]'s funeral; do you remember?

LETBY: Yes.

JOHNSON: That was in November 2015; yes?

LETBY: I don't recall the exact date.

JOHNSON: At that time you were living in Ash House?

LETBY: Yes.

JOHNSON: And for those of us that don't know the layout of the Countess of Chester site, how far is Ash House from the neonatal unit?

LETBY: Five minutes.

JOHNSON: Is it actually on the hospital estate?

LETBY: Yes.

MR JUSTICE GOSS: Five minutes' walk you mean?

LETBY: Sorry, yes.

MR JUSTICE GOSS: I think that's what we understood it to be.

MR JOHNSON: It's my fault for not clarifying.

Where did you write the card?

LETBY: I can't recall that specifically now.

JOHNSON: Well, you obviously went to a shop, I assume, or do you keep a stock of condolence cards?

LETBY: No, I would have had to have bought it.

JOHNSON: Right. That would have been when you weren't in work; is that right?

LETBY: Potentially, yes.

JOHNSON: Well, can you buy them in the hospital?

LETBY: No. Well, I don't know.

JOHNSON: Have you ever tried?

LETBY: No.

JOHNSON: Okay. So is the answer then: yes, I bought it when I wasn't in work?

LETBY: Yes.

JOHNSON: Thank you. And you will have taken it back to your accommodation, to Ash House?

LETBY: Yes.

JOHNSON: And you will have written it at your accommodation in Ash House?

LETBY: I can't recall specifically when it was written.

JOHNSON: Well, do you write cards in work on such a sensitive issue?

LETBY: I can't say. This card was going to work to be given to a member of staff. I don't know when I wrote the card.

JOHNSON: Have you got time to write condolence cards in your busy life as a neonatal nurse?

LETBY: I wouldn't have written it on shift, no.

JOHNSON: No. So my question then is: why do you take a picture of it on shift if you don't write it on shift?

LETBY: I don't know entirely when it was written, but is this taken on shift, the photograph?

JOHNSON: Yes, 4.38 in the morning on one of your shifts.

LETBY: Okay. So the card is written, it's come to work to be given to the staff that morning that are going to be funeral.

JOHNSON: The question that I invite you to answer is: if this is a card you've written at home, why do you take the picture at the place that [Baby I] died?

LETBY: I take photographs of the majority of cards and letters that I send regardless of the location.

JOHNSON: You are avoiding the question, Lucy Letby. [Baby I] died in dreadful circumstances, didn't she?

LETBY: Yes.

JOHNSON: Why did you take a picture of the card that you wrote to her parents at the place she died?

LETBY: Well, I can't say definitively 100% that is where the card was written.

JOHNSON: It was, there's GPS coordinates.

LETBY: And that can't be my home address at that time?

JOHNSON: I don't believe so. The evidence is it's the neonatal unit. So you tell us why you take a picture of a card, addressed to the parents of a child who has died in dreadful circumstances, at the place the child died.

LETBY: The place is insignificant. I take photographs of the majority of cards that I send. This was obviously taken into work to be given to the nurses attending the funeral. I've taken a picture, put it in an envelope, sealed it, and given it to staff.

JOHNSON: So it's another thing that's insignificant?

LETBY: No, I think that's out of context.

JOHNSON: No, no, I'm asking about the place, why you did it there, and your answer, you correct it if you want to --

LETBY: Wherever I take photographs is something that -- I don't think is relevant. It's the fact that my usual behaviour is that I photograph things that I send or receive.

JOHNSON: Yes, you'd photograph it when you write it.

LETBY: I can't say that, no. I don't know that I photograph it the minute that I've written something.

JOHNSON: Did it give you a bit of a thrill, photographing it at the place that this poor unfortunate child had died?

LETBY: Absolutely not, no.

JOHNSON: I want to clarify something with you about the nursing records. From time to time during your evidence, you have been referred to the lack of any mention of you in a specific child's paperwork. Do you recall that happening from time to time?

LETBY: In the sequence of -- in the -- yes.

JOHNSON: As Mr Myers asked you questions, he would refer you either to the sequence or to the neonatal review --

LETBY: Yes.

JOHNSON: -- and point out to you, and I'm not suggesting he's doing anything wrong at all, to make the point that your name doesn't appear on a specific child's paperwork.

LETBY: Yes.

JOHNSON: The point of that is that if you're not on the paperwork, you have had no contact with the child. Have I understood the point of that correctly?

LETBY: Yes.

JOHNSON: Yes. So just so that I understand, and so it may influence the questions I ask you, are you suggesting that the absence of your name appearing on a specific child's paperwork in effect shows that you haven't had contact with that child?

LETBY: It shows I've had no contact in terms of documentation that's required at that time. We regularly attend to babies if they're crying, unsettled, things like that.

That would not be in the paperwork.

JOHNSON: Did you ever go into the neonatal unit late at night when you weren't on shift?

LETBY: I have been to the unit on days off, yes.

JOHNSON: Why?

LETBY: At times I went to see colleagues when I was on the intensive care course or sometimes I've gone to finish documentation that hasn't been done in the day.

JOHNSON: So there are occasions, aren't there, Lucy Letby, that you were in that unit and there is no trace of you having been there?

LETBY: There would be a trace because I would have to have swiped to enter the unit. There's no other access.

JOHNSON: Are you sure about that?

LETBY: Yes.

JOHNSON: Well, let's look at the [Baby G] sequence of events, the first one, please. Tile 353:

"Just left work. Last gas 7.0. Lactate 9."

That's you to [Nurse A], isn't it?

LETBY: Yes.

JOHNSON: You were not on shift, were you, at 22.59 on Monday, 7 September 2015?

LETBY: I can't answer that without seeing the shift pattern.

JOHNSON: Well, let's look at the shift pattern. I'm not asking Mr Murphy to put this up on the screen because we have, of course, a paper copy behind divider 23 in jury bundle 2. It's page 3.

If we look in the bottom half of the page, you have the September work pattern; is that right?

LETBY: Yes.

JOHNSON: Were you working at 22.59 on Monday, 7 September?

LETBY: No.

JOHNSON: This, of course, was the day that [Baby G] had projectile vomited out of the cot; is that right?

LETBY: Yes.

JOHNSON: 7 September. You had finished work, officially at least, at 8 am that morning?

LETBY: Yes.

JOHNSON: As a matter of fact, you had been in the unit until 10 o'clock that morning, hadn't you?

LETBY: Yes, if that's agreed, yes.

JOHNSON: Well, if Mr Murphy would help me, please, by going to tile 185. That's a text you sent to your friend Jennifer Jones-Key at about 2.15 that afternoon --

LETBY: Yes.

JOHNSON: -- where you tell her that you didn't leave work until 10.

LETBY: Yes.

JOHNSON: So if that's true, it suggests, doesn't it, that you were in the unit for a couple of hours after your shift had ended?

LETBY: Yes. And that's not unusual.

JOHNSON: I'm not suggesting it is. I'm just trying to get to the truth. If I suggest something's unusual, I'll tell you, okay?

LETBY: Okay.

JOHNSON: If we go back to tile 351, please. You'd been having a look at [Baby G], hadn't you?

LETBY: Yes, I'd gone back into work to complete some documentation.

JOHNSON: Concentrate on the question if you don't mind: you'd been having a look at [Baby G], hadn't you?

LETBY: Yes, I have seen [Baby G], yes.

JOHNSON: If the purpose of going back in is to complete some documentation, why were you looking at this child?

LETBY: The documentation would have been in relation to [Baby G].

She'd been a sick baby. If you're in the unit I would go and check on her. I think that's not unreasonable.

JOHNSON: If we go to the next one, please. That's [Nurse A] telling you that [Baby G] was going to Arrowe Park, which we heard in evidence is true.

LETBY: Yes.

JOHNSON: The next one. That's the one I showed you a moment ago --

LETBY: Yes.

JOHNSON: -- which broadly reflects -- it's not exactly the same, but it broadly reflects [Baby G]'s last gas before you sent that message.

LETBY: Yes.

JOHNSON: That's what it was intended by you to do, wasn't it? So you'd been looking at her charts?

LETBY: Yes, or I've asked the member of staff looking after her for an update.

JOHNSON: 355, [Nurse A]'s response. And then on to 357, please.

[Nurse A] saying she had a bad feeling. Your response:

"Base excess..."

So another reference to the gas chart?

LETBY: Yes.

JOHNSON: "... looking good."

I think actually you're a couple of points out, but the gist of what you are saying is correct.

Then 364. That's [Nurse A]'s response. You next at 365:

"On today of all days."

Of course this was day 100.

LETBY: Yes.

JOHNSON: A big day for [Baby G]?

LETBY: Yes.

JOHNSON: 366. That's [Nurse A]'s response. 367. She refers to the parents. 368:

"She's declining bit by bit."

That was your view, was it?

LETBY: Yes.

JOHNSON: 369. Then your response at 370.

There is no record, of course, of you going into the unit on the door swipe data, is there?

LETBY: I can't answer that. I haven't looked.

JOHNSON: Well, there isn't.

LETBY: Okay.

JOHNSON: I'm not going to go through the whole sequence to prove the fact that something isn't in there. I'm sure I'll be corrected. It may be in that period of time where there is no door swipe data. But as a matter of fact, I'm going to suggest to you that you wouldn't need a pass to get in, would you? You wouldn't?

LETBY: Yes --

JOHNSON: Well --

LETBY: -- unless another colleague opened the door for me.

JOHNSON: You could ring the buzzer and say, "I've just come to sort something out", and walk in?

LETBY: Yes.

JOHNSON: Yes, and your presence would be accepted as a matter of course, wouldn't it?

LETBY: If I had legitimate reason for going, yes.

JOHNSON: People trusted you though, didn't they? Didn't your colleagues trust you?

LETBY: Yes, but to go to the unit at night, you have a reason to go.

JOHNSON: My question, though, is why did you choose to go at night, in the middle of the night, rather than during the day?

LETBY: Because I'd been on a night shift.

JOHNSON: Yes.

LETBY: So I've gone in the following evening, after having been in bed, and finished some of the paperwork.

JOHNSON: You went in at midnight. Would you usually do that?

LETBY: I don't think I went in at midnight.

JOHNSON: Sorry, 11 o'clock, I beg your pardon.

LETBY: If you go in at night-time -- I've just gone in at that time. I don't think that's relevant. It's quieter at night. I don't know, I can't say why I've gone in at that time.

MR JOHNSON: My Lord, that's a good hour.

MR JUSTICE GOSS: Certainly. We'll have a 15-minute break now, please, members of the jury.

(11.33 am)

(A short break)

(11.48 am)

MR JOHNSON: I just want to make sure -- well, first of all, is there anything that you've said before the break that we've just had that you would like to revise or amend?

LETBY: No.

JOHNSON: I just want to check that I have understood something that we've heard a bit about from various witnesses. It relates to the issue of NGT feeds; okay?

LETBY: Okay.

JOHNSON: Perhaps, rather than me asking you questions, can you just describe exactly how it's done?

LETBY: How we give a tube feed?

JOHNSON: Yes.

LETBY: We would have to get the milk out of the fridge, measure that into a separate pot to then be warmed. Once the milk is warmed, it would be put into a 10ml syringe and that 10ml

syringe would be attached to the baby's NG tube. Prior to that we would have aspirated the tube to check for the pH to make sure it's in the correct place.

Once we're happy the tube is right, we would then let the feed gravity fall into the baby.

JOHNSON: Have you ever used the plunger, a syringe plunger, to speed up the flow of milk?

LETBY: No.

JOHNSON: Is it a job for which you need to use both hands?

LETBY: Yes.

JOHNSON: Have you ever sent texts to your friends while you have been performing a tube feed?

LETBY: No, absolutely not, no.

JOHNSON: That would be -- where would that fall on the scale of infractions?

LETBY: Well, it's inappropriate and I don't see how you could do a feed without having both of your hands with the baby.

JOHNSON: Yes. So would it follow that if we can identify occasions where you are recorded as giving feeds but you're also sending texts to your friends, that you weren't really giving the feed?

LETBY: No. The feed charts are all estimated times, so I don't agree with that.

JOHNSON: Oh right. Tell us about the estimates.

LETBY: So feed charts are sort of done to the nearest quarter of an hour or half past the hour.

JOHNSON: So an extra half an hour? Earlier I think you said you went to quarters. Now you're giving yourself a bit more room.

LETBY: I just said quarters. I said quarter past or half past the hour.

JOHNSON: What about quarter to the hour?

LETBY: And quarter to the hour.

JOHNSON: And on the hour?

LETBY: Yes.

JOHNSON: So quarters is the way you would do it. And is the time -- is the nearest quarter the time that you get the milk out of the fridge or is it the time you actually start the feed?

LETBY: Usually it's the time you start the feed.

JOHNSON: Right. Have you ever used your phone in a clinical area?

LETBY: Not in the nurseries, no.

JOHNSON: What would take priority from your point of view, texting your friends or feeding a child?

LETBY: The baby, obviously.

JOHNSON: Have you ever texted your friends whilst a resuscitation is going on on the unit?

LETBY: No.

JOHNSON: Are you sure about that?

LETBY: A resuscitation that I've been involved with?

JOHNSON: Whilst a resuscitation's going on on on the unit.

LETBY: Not that I can recall, no.

JOHNSON: Giving anyone a commentary as to what's going on?

LETBY: No.

JOHNSON: Never? It'd be wholly inappropriate, that sort of thing, would it?

LETBY: I can't recall texting during a resuscitation, no.

JOHNSON: Can we have an answer to the question: would it be wholly inappropriate?

LETBY: Yes, if I was at the cot side, yes.

JOHNSON: But not if you're somewhere else in the unit?

LETBY: No, it's accepted that we use our phones in other places on the unit, yes.

JOHNSON: But that's not the question, is it? The question is: is it appropriate to be texting your friends whilst a resuscitation is going on?

LETBY: If I'm not playing a part in that then, yes.

JOHNSON: That's fine, yes, giving a commentary to your friends?

LETBY: Well, it's not a commentary.

JOHNSON: Right. Do you know what I'm talking about?

LETBY: No.

JOHNSON: Well, we'll come to it.

Staffing levels. You're not suggesting, are you, that the staffing levels in the unit remained the same irrespective of the number of children that were in the unit?

LETBY: Yes, the staffing levels are set standard levels.

JOHNSON: Set standard levels, but they would vary depending on how many babies were in the unit?

LETBY: At times, yes.

JOHNSON: And what the needs of the children were?

LETBY: Yes.

JOHNSON: We'll deal with specifics in due course, but yesterday, for example, you were being asked about the number of children in a particular case that were in nursery 1.

And you suggested that they were all intensive care babies. That's not necessarily true, is it?

LETBY: I don't know who you're talking about.

JOHNSON: Well, we will come to specifics, but I'm asking you general questions at the moment. Don't worry, you will be able to deal with the specifics, but what I am asking you to do is to deal with generalities first of all.

Now, we heard from several witnesses during the course of the evidence that just because a baby is in nursery 1, it doesn't follow that they are an intensive care baby.

LETBY: No, that's right.

JOHNSON: So that's correct. It also follows, doesn't it, that just because a baby is in nursery 2, they are not a high dependency baby necessarily?

LETBY: No, that's right.

JOHNSON: So it's dangerous to look at the location of a child and conclude necessarily that their needs are at a particular level; do you agree?

LETBY: Yes, you'd have to look at the context of the baby, yes.

JOHNSON: Exactly. Do you agree that if certain combinations of these children were attacked then unless there was more than one person attacking them, you have to be the attacker?

LETBY: No.

JOHNSON: You don't agree?

LETBY: No. I've not attacked any children.

JOHNSON: I understand your case, you are saying you haven't done anything. Right? But if the jury conclude that a certain combination of children were actually attacked by someone, then the shift pattern gives us the answer as to who the attacker was, doesn't it?

LETBY: No, I don't agree.

JOHNSON: You don't agree. Why don't you agree?

LETBY: Because just because I was on shift doesn't mean that I have done anything.

JOHNSON: No, but it would follow that if the -- let's say -- I'll use numbers, all right? I won't refer to specific cases. Let's say if baby 5, 8, 10 and 12 were all attacked, if the jury look at the medical evidence and say they were all attacked by someone, and you're the only common feature, it would have to be, wouldn't it, that you're the attacker?

LETBY: That's for them to decide.

JOHNSON: Well, of course it is, of course it is. But as a principle, do you agree with that?

LETBY: No, I don't feel I can answer that.

JOHNSON: Okay. I am going to ask you about some of the doctors individually, all right?

LETBY: Yes.

JOHNSON: I'll deal with them in alphabetical order. Lucy Beebe, who was at the time a senior house officer, who reviewed [Baby I]. Did you get on well with her?

LETBY: No, I don't recall Lucy.

JOHNSON: You just don't remember her at all?

LETBY: I remember when she came to evidence, I recognised her, but other than that, no.

JOHNSON: She doesn't bear you a grudge as far as you know?

LETBY: No.

JOHNSON: You never fell out with her as far as you remember?

LETBY: No.

JOHNSON: Dr Gail Beech who dealt with the [Surname of Babies E & F] boys.

LETBY: Yes.

JOHNSON: Do you remember her?

LETBY: Yes.

JOHNSON: Did you ever have any problems with Dr Beech?

LETBY: No.

JOHNSON: So far as you know, does she bear you any ill will?

LETBY: No.

JOHNSON: Dr Stephen Brearey, did you ever have any problems with him?

LETBY: No.

JOHNSON: I think a while ago, you said he was "one of the bastards".

LETBY: At the time that we're talking about when I was at work on the unit, I did not have a problem with Steve then, no. It came after.

JOHNSON: Okay. You were asked, I think, by Mr Myers why you categorised him in that way; do you remember?

LETBY: Yes.

JOHNSON: And you said you were putting him in the same category as Dr Jayaram, Dr Ravi Jayaram, he said or you said, and when asked the question, you said:

"Because of the things they'd been saying about [you]."

That was the reason you categorised them in that unflattering way.

LETBY: Yes.

JOHNSON: What had they been saying about you?

LETBY: They had been making comments that I was responsible for the deaths of babies.

JOHNSON: Right. Anything else? Had they said anything else that made you categorise them --

LETBY: Yes, that they were very insistent that I be removed from the unit.

JOHNSON: So far as while you were working on the unit until you were removed from the unit at the end of June, you hadn't had a problem with him, is that fair or not?

Well, let's stick with one by one: Dr Brearey, you didn't have -
-

LETBY: We just had a normal working relationship, yes.

JOHNSON: Dr Andrew Brunton, the Scotsman who dealt with [Baby D].

LETBY: Yes.

JOHNSON: Any problems with him?

LETBY: No.

JOHNSON: Dr Rachel Chang, who dealt with the [Surname of Babies O, P & R] boys?

LETBY: No, no problem.

JOHNSON: What about [Dr A]? Any problem with him?

LETBY: No.

JOHNSON: Were you in love with him?

LETBY: No.

JOHNSON: Did you ever write down that you were in love with him?

LETBY: No. I loved [Dr A] as a friend; I was not in love with him.

JOHNSON: Can we look at image 10, please. I'm not going to deal with much of your writings at this stage; okay? I'll deal with them at the end. But I just want to ask you about this one.

This, the jury will remember, is the note addressed to the [Surname of Babies O, P & R] triplets; is that right?

LETBY: It refers to [the triplets], yes.

JOHNSON: Yes. Well, you took us through the note to them and then you filled in the gaps with names and other words; is that right?

LETBY: Yes.

JOHNSON: You see on the left-hand side of the screen there's a very heavy circle.

LETBY: Yes.

JOHNSON: You see just above that, it says "I'm sorry".

LETBY: Yes.

JOHNSON: And you see just above the word "sorry" there is a word; got that?

LETBY: Can I move the cursor?

JOHNSON: Yes, please. Please do.

LETBY: This word (indicating)?

JOHNSON: That word, yes. Mr Myers suggested to you that that said "Timmy"; do you remember?

LETBY: Yes.

JOHNSON: And you said that was a reference to a dog you had as a child?

LETBY: Not that I had as a child, a dog that I used to walk at that time, yes, called Tim.

JOHNSON: What it actually says is "tiny boy", doesn't it?

LETBY: I can't confirm either way. It does look like "boy", yes.

JOHNSON: Tiny boy. Who was tiny boy?

LETBY: My dog as a child called Whiskey. We used to refer to him as tiny boy.

JOHNSON: So it doesn't say Timmy?

LETBY: I can't say specifically. It could have been a reference to Timmy or it could be tiny boy. Either way, it's about the dog.

JOHNSON: Well, we'll come to -- we'll remember what you've just said and we'll come to more references in due course on other documents. We'll see.

Moving on, if we can remove the image, please.

Thank you. Dr Peter Fielding, who was involved with [Baby G].

LETBY: Yes.

JOHNSON: Any issues with Dr Fielding?

LETBY: No.

JOHNSON: Dr John Gibbs.

LETBY: Yes.

JOHNSON: Any issues with Dr Gibbs?

LETBY: No.

JOHNSON: Dr David Harkness. Any issues with him?

LETBY: No.

JOHNSON: [Dr B]. Any issues with [Dr B]?

LETBY: We didn't have the best working relationship, but we worked amicably together.

JOHNSON: Well, you didn't approve of her smoking; is that fair?

LETBY: Yes.

JOHNSON: She probably wouldn't have approved of you taking handover sheets home if she'd known about it, would she?

LETBY: That's something for her to answer.

JOHNSON: Yes. Dr Jayaram. Is there any additional information you want to give us about him other than what you've already told us?

LETBY: No. At the time of these events we had a normal working relationship.

JOHNSON: You were very interested in him, weren't you, on the internet?

LETBY: Sorry?

JOHNSON: You were very interested in him on the internet, he was one of the people you'd search --

LETBY: I search for a lot of people, yes.

JOHNSON: You did. Dr Rachel Lambie, who dealt with [Baby B].

LETBY: Yes.

JOHNSON: Any issues with her?

LETBY: No.

JOHNSON: Dr Elizabeth Newby, one of the consultants?

LETBY: No.

JOHNSON: No problem?

LETBY: No.

JOHNSON: Dr Sally Ogden?

LETBY: No.

JOHNSON: Dr Sarah Rylance? She was the lady who gave evidence from Switzerland, you may remember, in the case of [Baby D].

LETBY: No, there's no issue.

JOHNSON: Dr Saladi?

LETBY: No.

JOHNSON: [Dr C]?

LETBY: No.

JOHNSON: Dr Alison Ventress?

LETBY: No.

JOHNSON: Finally, Dr Christopher Wood. He was the SHO who you claim to have spoken to in the case of [Baby E].

LETBY: Yes.

JOHNSON: Any problems with him?

LETBY: No.

JOHNSON: Are you suggesting that there is some sort of agreement between any of the medical staff who have given evidence in this case, some sort of agreement to get you?

LETBY: In a consultant group, yes, I do believe that.

JOHNSON: Who's in the conspiracy group then?

LETBY: Which individuals?

JOHNSON: Oh yes.

LETBY: I believe Ravi Jayaram, Stephen Brearey, [Dr B] and John Gibbs.

JOHNSON: So the Gang of Four; yes?

LETBY: Yes, possibly with input from --

JOHNSON: I'll refer to them that way rather than repeating their names.

LETBY: Yes.

JOHNSON: And what is the conspiracy between the Gang of Four?

LETBY: That they have apportioned blame on to me.

JOHNSON: And the motive for apportioning blame on to you is what, please?

LETBY: I believe to cover failings at the hospital.

JOHNSON: Right. Well, just so that you can be thinking about it, in each case I will ask you questions so that you can explain to the jury what the failings were; all right?

LETBY: Okay.

JOHNSON: By and large you haven't been asked those questions yet, but I will ask them. I'm not going to ask you now as a general point because it's not sufficiently precise.

What I will do, though, is ask you in each case what the failings are. So would you think about that so that you can answer the questions in due course?

LETBY: Yes, I will.

JOHNSON: Thank you. Do you agree that [Baby F] was poisoned with insulin?

LETBY: Yes, I agree that he had insulin, yes.

JOHNSON: Do you agree that somebody gave it to him unlawfully?

LETBY: Yes.

JOHNSON: Do you agree that somebody targeted him specifically?

LETBY: No.

JOHNSON: So you think it was a random act?

LETBY: I don't know where the insulin came from.

JOHNSON: Do you agree that [Baby L] was poisoned with insulin?

LETBY: From the blood results, yes.

JOHNSON: Do you agree that somebody targeted him specifically?

LETBY: No. I don't know that the intent was to cause harm.

I don't know how the insulin got there or by who.

JOHNSON: What are the possibilities, realistically? Knowing what you know as a senior nurse, what are the possibilities as to how

that insulin got into his dextrose -- I'm talking about [Baby L] now -- bag?

LETBY: It's happened at some point on the unit or the bag already had the insulin in it when it came up to the unit from pharmacy.

JOHNSON: Well, we will deal specifically with the evidence relating to [Baby L] and I will invite you in due course to reconsider that answer; all right? But your answer for the time being is either it was in the bag when it came up from the pharmacy or someone's added it on the unit?

LETBY: You've asked me ways that it could happen, yes.

JOHNSON: Yes, I'll be a bit more specific then: knowing what you know about the insulin readings or the blood sugar readings, to be more precise, what are the realistic possibilities in [Baby L]'s case as you see it?

LETBY: I don't believe that any member of staff on the unit would make a mistake in giving insulin.

JOHNSON: No. Mistake is not an option in this case for the insulin babies, is it?

LETBY: No.

JOHNSON: No.

MR JUSTICE GOSS: That's for [Baby F] as well, for [Baby F] as well, you accept that?

LETBY: Yes.

MR JOHNSON: So it's deliberate poisoning by someone, but not you?

LETBY: Insulin has been added by somebody. I can't comment on how or who.

JOHNSON: Right.

LETBY: Just that it was not me.

JOHNSON: What about when?

LETBY: When what?

JOHNSON: Well, again, I will come back to this; all right? Let's take [Baby L]. When was the insulin first put in [Baby L]'s dextrose bag?

LETBY: Well, I don't know because I didn't put insulin in, so I've no idea.

JOHNSON: But what do the blood sugar results tell us as to when it was put in?

LETBY: I can't recall now without looking at the charts.

JOHNSON: All right. Well, I'll come back to that. Park that for now.

Have you not given that any thought as to when it's happened?

LETBY: Yes, but I haven't got the details in front of me right now.

JOHNSON: Okay. I will always give you the opportunity to look at documents, don't worry. Don't feel under any pressure from me to answer any question without looking at the documents if you feel you need to look at them. All right?

LETBY: Okay.

JOHNSON: But what are the dangers of unprescribed insulin?

LETBY: Insulin should not be given to a patient that doesn't need it.

JOHNSON: Yes, but what's likely to happen if it is given to a patient who doesn't need it?

LETBY: It will cause them to become unwell. They will become hypoglycaemic and with that comes a number of problems such as seizures, apnoeas, even death.

JOHNSON: I'm going to move on to your QIS training; all right?

LETBY: Yes.

JOHNSON: This is the last subject I'm going to deal with before I turn to [Baby A]. I will deal with the babies in the same order that Mr Myers did to help you. So that's the menu.

You started your training towards the end of 2014; is that right?

LETBY: I can't remember now, but if that's agreed then, yes.

JOHNSON: It's what you said on 2 May.

LETBY: Okay.

JOHNSON: So you seemed to remember it on 2 May.

LETBY: Okay.

JOHNSON: If anyone wants to check what you said, if they look at page 32 of that transcript, they will find that that is what you said.

I think you told the jury that it involved a university module, a placement at the Women's Hospital in Liverpool and it took about 6 months.

LETBY: Yes.

JOHNSON: And that you couldn't be specific as to the date that you qualified but, as you remembered it, it was March/April of 2014; is that right?

LETBY: 2015.

JOHNSON: I beg your pardon. My mistake.

When you were asked to summarise what the consequence of the qualification was, you said it meant that you were qualified to look after intensive care babies.

LETBY: Yes.

JOHNSON: And in practical terms, so far as your day-to-day employment at Chester was concerned, did that have the effect of putting you into nursery 1 much more often than you would have been without the qualification?

LETBY: Yes.

JOHNSON: Subject, of course, to the point that we've already made about the fact that a child in nursery 1 isn't always intensive care.

LETBY: No.

JOHNSON: Part of your training involved education about lines?

LETBY: Yes.

JOHNSON: And as we saw in the paperwork which was adduced right at the end of the prosecution case, one of the issues with line access is the possibility of air embolus?

LETBY: Yes.

JOHNSON: And so at the time that you dealt with [Baby A] and [Baby B] at the beginning of June 2015, how long had you been qualified to access the lines of neonates?

LETBY: So after the ITU course you can access central lines, so prior to that it would have been peripheral lines only.

JOHNSON: You had, of course, heard of air embolus, hadn't you, when you were spoken to by the police?

LETBY: Yes.

JOHNSON: And what did you understand from your education and wider knowledge were the dangers of injecting air into a line?

LETBY: I think all nursing staff know that any air getting into a patient could ultimately lead to death.

JOHNSON: Yes. Even in the films, when a doctor is about to give an injection to a patient, you see them tap the syringe and squirt a bit of liquid out at the end, don't you?

LETBY: Yes.

JOHNSON: So everybody knows the dangers of air embolus, don't they, pretty much?

LETBY: Yes, it's part of your nurse training, yes.

JOHNSON: But even people with no medical training like me, everybody knows, don't they?

LETBY: I can't speak for everyone.

JOHNSON: All right. Let's start with the case of [Baby A], please. You were first interviewed on 4 July 2018.

Independently of the medical notes, did you have any memory of [Baby A]?

LETBY: Yes.

JOHNSON: Just for anyone's note, it is, of course, count 1.

He was born on 7 June and died a day later on the 8th, and he was, as we all know, the twin of [Baby B].

Before [Baby A], had you ever known a child to die unexpectedly within about 24 hours of birth?

LETBY: I can't comment on that. I'm not sure.

JOHNSON: Well, how many infant deaths had you witnessed in your employment before [Baby A]?

LETBY: Not many, maybe two or three at the Countess and then several more on my placements at Liverpool Women's.

JOHNSON: Two more at Liverpool Women's? You say several more.

I'm offering an alternative of two more.

LETBY: I can't say. I don't keep track of numbers.

JOHNSON: I'm just going off what you said to the police, you see.

LETBY: Well, my memory then would have been clearer than it is now.

JOHNSON: Okay. Well, we can go to the interviews if you'd like to check it, page 16 of your [Baby A] interview.

I'm suggesting that you told the police that you'd seen two at Liverpool.

LETBY: Okay.

JOHNSON: Does that sound right?

LETBY: Yes.

JOHNSON: When [Baby A] and [Baby B] were born, were the staff, including the nursing staff, talking about their mother's antiphospholipid syndrome?

LETBY: Not at the time -- when I was looking after [Baby A]?

JOHNSON: At any time. You tell us.

LETBY: Well, the first time I met [Baby A] and was on shift was the day of the event. Afterwards, the condition was talked about, yes.

JOHNSON: Was it being offered up as an explanation for what had happened as far as you were aware?

LETBY: It was possibly a contributing factor. That was what was being discussed at the time.

JOHNSON: But we now know that it wasn't, don't we?

LETBY: Yes.

JOHNSON: Is it your case that the staffing levels in the Countess of Chester Hospital contributed to [Baby A]'s death?

LETBY: In part, yes.

JOHNSON: Right. How?

LETBY: Through the amount of time that he was without fluids for and the issues around the lines' insertion.

JOHNSON: Right. So he was without fluids from some time about 4 pm or thereabouts, 3 or 4 pm?

LETBY: Yes.

JOHNSON: So your case is, what, that he died from dehydration?

LETBY: No. I'm saying they're contributing factors.

JOHNSON: But according to you, at least, how did that lack of fluids for about 4 hours contribute to his death?

What's your case?

LETBY: That it would have put him at increased risk of collapse and he would have had less reserves to then fight off any infection or any procedures that we were doing.

JOHNSON: So it's your case that 4 hours without fluids killed [Baby A]?

LETBY: No, that's not what I'm saying, no.

JOHNSON: What --

LETBY: I can't tell you how [Baby A] died. I don't know --

JOHNSON: Okay.

LETBY: -- but they're contributing factors that were missed.

JOHNSON: Is it your case that the competence of the medical staff contributed to [Baby A]'s death?

LETBY: Again, I can't say definitively how [Baby A] died but, yes, I think the issues around his lines made him more vulnerable, yes.

JOHNSON: And what issue around his lines made him more vulnerable?

LETBY: That he was left with a line in situ, a UVC, that was not for use and was due to be removed and it wasn't.

And then having a long line sited and time passing by without that line being connected to anything and being confirmed on X-ray. It's not usual practice.

JOHNSON: All right. Did you fill in a Datix?

LETBY: No.

JOHNSON: Why not?

LETBY: I just haven't.

JOHNSON: You fill in lots of Datix forms, don't you?

LETBY: Yes.

JOHNSON: Three at the end of June 2016.

LETBY: Right.

JOHNSON: We'll come to them, but two of which contained lies, according to Dr Brearey; yes?

LETBY: According to Dr Brearey.

JOHNSON: Yes. If you seriously thought that [Baby A]'s death was contributed to by him not having fluids for any significant period of time, why didn't you fill in a Datix form?

LETBY: It was discussed amongst staff, we all talked about that as contributing factors at the time. That was readily discussed amongst staff, including the consultant.

JOHNSON: No, no, the question is, why did you --

LETBY: I didn't feel I needed to do a Datix. It had been raised verbally by senior staff -- to senior staff.

JOHNSON: And those senior staff were who?

LETBY: Ravi Jayaram and other members of the nursing team.

JOHNSON: No, Ravi Jayaram and?

LETBY: I didn't say two senior staff: I escalated the concerns to senior staff --

JOHNSON: Oh, I beg your pardon.

LETBY: -- one being Ravi Jayaram, the other senior nursing staff.

JOHNSON: I'm sorry, I misunderstood what you said, I apologise.

LETBY: And part of that discussion is why we then kept the fluids and the line from [Baby A].

JOHNSON: I'll come to that in a moment, if I may. Was [Baby A]'s death anything to do with Mel Taylor?

LETBY: I don't know why [Baby A] died.

JOHNSON: But I'm asking you whether it's your case that it had anything to do with Mel Taylor.

LETBY: If we agree that the cause of death was an air embolus then I believe she attached fluids to those lines, yes.

JOHNSON: So that's Mel Taylor's fault?

LETBY: Yes, if that's what we agree, that [Baby A] has died from that, then yes.

JOHNSON: Because you weren't standing over him at the time he collapsed?

LETBY: No, I did not have any access to his lines, so therefore if an embolism came, it must have come from the person connecting the fluids, which is not me.

JOHNSON: Would you admit it if you did?

LETBY: Did what?

JOHNSON: Put air into him.

LETBY: I haven't put air in so I am not going to --

JOHNSON: The question is: would you admit it if you did?

LETBY: It's not something I would ever do.

JOHNSON: I'm going to ask you about some of the descriptions that have been given by your colleagues in evidence about [Baby A] to see whether you agree or disagree with them.

All right?

LETBY: Okay.

JOHNSON: Starting with Dr Beech's evidence. On 7 June, Dr Beech told the jury that [Baby A] was doing well, that he was crying and vigorous on stimulation shortly after his birth at about 9 pm, 21.00 hours, on 7 June. Do you disagree with that?

LETBY: No, I wasn't present for that, so...

JOHNSON: The following day, the day he died, which was Monday, 8 June, at 09.45 Dr Ogden, at tile 84, described [Baby A] as "handling well". Do you disagree with that?

LETBY: Again, I wasn't there so I can't comment. That's her opinion.

JOHNSON: Well, are you aware of anything to contradict --

LETBY: No.

JOHNSON: -- Dr Ogden's opinion?

LETBY: No.

JOHNSON: And as you've already told us, in the afternoon there were problems identified with the UVC and then the cannula to which he was attached became blocked?

LETBY: Yes.

JOHNSON: Do you remember that at 19.00 hours on 8 June, Dr Harkness put a central line into [Baby A]?

LETBY: I know that happened, yes.

JOHNSON: Do you accept that you were standing over [Baby A] at the time he collapsed?

LETBY: I accept that I was within his cot space checking equipment, yes.

JOHNSON: How close were you to him at the time he collapsed?

LETBY: I was in the close vicinity, I was checking the equipment around his incubator.

JOHNSON: Yes, give us an idea in terms of this court. Could you reach out and touch him?

LETBY: I could touch the incubator, not -- I'd have to open the doors to actually get into the baby, but yes I could touch the outside of the incubator.

JOHNSON: Could you touch the lines?

LETBY: No.

JOHNSON: Why not?

LETBY: The incubator was closed.

JOHNSON: Do you accept that at the time [Baby A] collapsed, Mel Taylor was on the computer doing her nursing notes?

LETBY: Yes.

JOHNSON: It's your case, and correct me if I have misunderstood, that Mel Taylor was the sterile nurse in terms of attaching [Baby A]'s line.

LETBY: Yes.

JOHNSON: Do you accept that it was at 20.05 hours that the bag was hung?

LETBY: Yes.

JOHNSON: Do you accept that you co-signed for that?

LETBY: Yes.

JOHNSON: And do you accept that the order of the signatures doesn't illuminate the question of who it was that actually hung --

LETBY: That's correct, there's no way of knowing from signatures.

JOHNSON: It was 15 minutes later, wasn't it, that [Baby A] collapsed at 20.20?

LETBY: Yes.

JOHNSON: And what I'm going to do is to remind you of various people's descriptions of what they saw. Okay? What I'm going to ask you is whether you agree or disagree with their descriptions.

LETBY: Okay.

JOHNSON: Do you understand?

LETBY: Yes.

JOHNSON: So if you want me to repeat any of this, you say so; all right?

LETBY: Okay.

JOHNSON: I'm quoting from the transcript and I'm going to start with Dr Harkness. For your Lordship's note, this is Day 12, pages 90 to 92.

MR JUSTICE GOSS: Thank you.

MR JOHNSON: What Dr Harkness said was this:

"There was a very unusual patchiness of the skin, which I had never seen before and I have only seen in cases since within the Countess of Chester Hospital and nothing other than that."

I'm not asking you to comment on what he has or hasn't -- what he says about having seen things, but do you agree with his description that there was a very unusual patchiness of his skin?

LETBY: No.

JOHNSON: "There were patches of a kind of blue/purple colour, there were patches of bright red colour, there were white patches and they didn't fit with something that you would find on a baby that's not pumping any blood around their body."

Do you agree with that?

LETBY: No.

JOHNSON: "This had bright red patches, which does not fit because red patches means you've got blood going round your body."

Do you agree with that?

LETBY: No.

JOHNSON: "It had purple patches, it had white patches."

LETBY: I agree with that, yes, he had mottling.

JOHNSON: No red marks?

LETBY: No.

JOHNSON: No red blotchiness?

LETBY: Sitting here now, I can't recall any blotchiness, no.

JOHNSON: Are you saying Dr Harkness has made that up?

LETBY: I didn't see it. If he saw something that I didn't, that's something he has to justify.

JOHNSON: That's not quite the question, you see. I'm asking whether you dispute something. If you can't comment then that's one thing, but you say no. I said, "Is that right", "No", which --

LETBY: It's not something I saw.

JOHNSON: Right. Well, I'm not interested in, at the moment -- I will come to what you saw. The question is: are you disputing what Dr Harkness says he saw?

LETBY: Yes.

JOHNSON: Why?

LETBY: Because I was present and I did not see those.

JOHNSON: Right. So because you didn't see it, you are disputing it?

LETBY: Yes.

JOHNSON: Are you suggesting he's lying about it?

MR MYERS: My Lord, can I say at this point, each time now if we go to are they lying or not when the defendant has said she simply does not agree, it isn't fair or right to ask her to say each time whether that is because they are lying, whether they are mistaken or whatever.

That isn't a proper question, in my submission, to ask if they're lying in that situation.

MR JOHNSON: It's been put to the witnesses that they are lying. I want to know what the defendant's case is.

MR JUSTICE GOSS: I think that the point -- what the witness has said is that description that is given by Dr Harkness does not accord with what she saw.

MR JOHNSON: Yes.

MR JUSTICE GOSS: It's ultimately a matter for the jury as to whether he is lying or she is lying.

MR JOHNSON: Yes. Very well.

MR JUSTICE GOSS: Therefore I am not going to permit you to ask that question again.

MR JOHNSON: Very well.

Dr Jayaram. This was his description and this was on Day 14, page 126:

"He was pale. He was pale. What I really didn't give any clinical significance to at the time was sort of unusual patches of discolouration."

Do you agree with that?

LETBY: I don't agree with the discolouration, I agree he was pale.

JOHNSON: "It's interesting because it's the first time I've seen that. [Baby A] was very pale to blue."

Do you agree with the pale to blue?

LETBY: He was pale, I don't recall him being completely blue, no.

JOHNSON: "But there were unusual -- the best way to describe them is pink patches that appeared mainly on the torso, which seemed to sort of appear and disappear and flit around."

Do you agree with that?

LETBY: No, I don't.

JOHNSON: [Nurse A], your friend. She was your friend, wasn't she?

LETBY: Yes.

JOHNSON: A good friend?

LETBY: Yes.

JOHNSON: Do you trust [Nurse A]?

LETBY: As a colleague or as a friend?

JOHNSON: Both.

LETBY: Yes.

JOHNSON: What she said -- how many years' experience did [Nurse A] have?

LETBY: I don't know exact figures, but she was very experienced.

JOHNSON: Approximately. 20 years?

LETBY: Possibly, yes.

JOHNSON: Is she the sort of person you would defer to, whose advice you would take?

LETBY: Yes.

JOHNSON: If you had a problem, is she the sort of person that could be relied on to give you a good solution?

LETBY: Yes.

JOHNSON: What she said is this:

"I'd never seen a baby look that way before. I mean, he looked very ill."

Did he look very ill?

LETBY: Yes.

JOHNSON: "He had a discolouration pattern that I've never seen before."

Was this a discolouration pattern that you had ever seen before?

LETBY: I didn't see a discolouration pattern.

JOHNSON: So you don't agree with that?

LETBY: No.

JOHNSON: "He was very white with sort of purply blotches."

Do you agree with that?

LETBY: I agree that he was white with purple markings that looked like mottling, yes.

JOHNSON: "He did look cyanotic as well but it was this purply blotchiness with white that I had not seen before."

Do you agree with that?

LETBY: No.

JOHNSON: She is not describing there, is she, the common or garden blotchiness that babies have on neonatal units?

LETBY: No.

JOHNSON: Because she's never seen it before with all that experience; do you agree with that?

LETBY: That's what she said, yes.

JOHNSON: "My recollection, it was all over him, everywhere.

Maybe more torso, and I can't remember clearly but I just -- unusual, it had come on very suddenly."

Do you agree with that?

LETBY: I agree that the colour change came on very suddenly, yes.

JOHNSON: Let's go to what you said, bearing in mind that you don't accept the red marks. Can we go to your interview, please, and it's in the first interview bundle, it's the very first of the interviews, so it's [Baby A] number 1. If you just turn back a page from where you are, if you wouldn't mind, we'll just do the formalities.

We can see here that the date of this interview was 4 July. [Initials of Baby A] 1. Have you got that?

LETBY: Yes.

JOHNSON: 2018, I should have added. The time that the interview commenced was 10.23. It concluded at 12.04.

Let's go to page 8 and get your description, please.

You were being asked to describe what you saw; is that right?

LETBY: Yes. I'm asked to interpret what pale is, yes.

JOHNSON: Yes. So the second question on the page, you're asked:

"Question: Tell me how you interpret pale first of all in a neonatal baby.

"Answer: So it's a loss of colour: babies are usually quite pink and he'd become more pale as in almost white."

And you make it clear that that means there's something wrong, it could be infection, could be hypoglycaemia when they've had had a sudden collapse.

Then this question:

"Question: And mottled, the same thing, how do you interpret the mottled (inaudible) appearance?

"Answer: Mottled is a bit more of -- almost like a rash appearance, like blotchy red marks on the skin."

Is that what you were referring to as mottled?

LETBY: Yes.

JOHNSON: Then further down the page, you say:

"So when the baby's quite pale and white they can have sort of red areas on them."

Red; is that right?

LETBY: Yes, this is all in relation to mottling, yes.

JOHNSON: And you reply over the page:

"Yeah, like reddy/purple."

LETBY: Yes.

JOHNSON: You say he was centrally pale.

LETBY: Yes.

JOHNSON: Then if we go to page 11, please. Halfway down the page you're being asked about the time [Baby A] collapsed.

It's pretty much exactly halfway, you say:

"I believe I was checking the equipment, yes, at his cot side and his charts."

Is that right?

LETBY: Yes.

JOHNSON: Then the police officer asked you this:

"When you talk about this mottled rash, can you give us any further description of how it showed, shape?"

So what you're being asked here is, "What did you see", aren't you?

LETBY: Yes.

JOHNSON: You're not being asked about your description of mottling. You're being asked the specific question, "What did you see"; do you agree?

LETBY: In relation to the mottling, yes.

JOHNSON: No, in relation to what you saw on [Baby A]. Whatever word you want to --

LETBY: When you talk about this mottled rash, yes, that's what I'm referring to.

JOHNSON: What the police -- I'm paraphrasing it, but what the police are saying to you here is you've said he had a mottled rash --

LETBY: Yes.

JOHNSON: -- what did you see --

LETBY: Yes.

JOHNSON: -- what are you saying is a mottled rash; yes?

LETBY: Yes.

JOHNSON: Do you agree? Fair?

LETBY: Yes.

JOHNSON: So that's the question:

"Question: Can you give us any further description of it, how it showed, shape?

"Answer: I think from memory it was more on the side that had his line in."

Is that right?

LETBY: Yes, I remember the paleness and the discolouration was more to the left-hand side, yes.

JOHNSON: And:

"Question: Which side was that, do you remember?"

And you said, as you have just said:

"Answer: "I think it was his left.

"Question: And you described it as a red/purple colour. Can you sort of help us with any comparisons with anything colour-wise, the way it showed? Was there any -- was it bright red, was it dark red?

"Answer: I think he was more pale than mottling, than the little red areas, but predominantly it was pale [as read]."

Is that right?

LETBY: Mine doesn't say "red areas" here.

JOHNSON: Sorry, I may have misread something:

"I think he was more pale than the mottling, than the little areas of the mottling."

I beg your pardon.

LETBY: Yes.

JOHNSON: Were you not describing blotchy red marks on his skin?

LETBY: I was referring to mottling. That's how I would describe mottling and I also felt that he was more pale than mottled.

JOHNSON: So is it your evidence, just so that we understand where the lines between the prosecution and you are, is it your evidence that there was nothing unusual to be seen on [Baby A]?

LETBY: No, it's unusual to have a baby that is pale and he was particularly white on one side. That is unusual.

JOHNSON: So far as the discolouration was concerned, is it your case that there was nothing unusual about the discolouration?

LETBY: I don't recall there being an abnormal discolouration, no.

JOHNSON: So that's your case?

LETBY: Yes.

JOHNSON: So far as the testing of the bag was concerned, which you mentioned earlier, I think [Nurse A] accepted that you had asked for the bag to be tested, as you said in interview.

LETBY: Yes.

JOHNSON: Was it tested?

LETBY: I don't know. I put it aside and labelled it, the responsibility then went to the shift leader of the next shift.

JOHNSON: Did you ever follow it up?

LETBY: I -- I can't say. I can't recall specifically, no.

I know I labelled the bag and I put the request in the diary and I handed it over to the shift leader.

JOHNSON: Right. Further evidence that was given by those who tried to save [Baby A]. Dr Harkness. Do you remember him telling the jury how he bagged [Baby A]?

LETBY: I can't recall that now, but yes, if that's what he said.

JOHNSON: And he said that [Baby A]'s chest was rising and falling, that his heart rate and his saturations were declining; do you remember that?

LETBY: Yes. Well, not specifically, I'm relying that what -- that that's what he said. I can't recall everybody's evidence in detail.

JOHNSON: Sure. I'm sure I'll be corrected if I'm misrepresenting what people have said.

Do you accept, given that pattern of air going in and out, the sats and the heart rate falling, that this wasn't a normal respiratory problem that [Baby A] had?

LETBY: I don't think I can comment on what condition [Baby A] had at that point.

MR JUSTICE GOSS: I don't think that's quite the question.

MR JOHNSON: No.

LETBY: Sorry, I don't understand then.

MR JUSTICE GOSS: Can you ask the question again? That's not an answer to the question.

MR JOHNSON: I'm not asking you for a diagnosis, what I'm asking you to give us your point of view about is the issue of air going in and out and the heart rate falling and the saturations falling. So the evidence is that air is going in, carbon dioxide is going out with more air, but more carbon dioxide, and yet [Baby A]'s heart rate and saturation rate were falling. And that is not a normal respiratory problem, is it? If air --

LETBY: No, generally if you've got an airway and you've got chest movement, you would expect to see a rise in the observations, yes.

JOHNSON: Exactly. Dr Jayaram dealt with a number of other issues, which I'm asking for your comment on, because you were there. He said that the heart rate on the Philips monitor was normal; do you agree?

LETBY: I can't recall specific values now.

JOHNSON: But if there had been something abnormal, would you have taken any notice of it?

LETBY: His monitor did alarm. That was how we were alerted to him having a desaturation, so he must have had a change in observations.

JOHNSON: Yes. The trace -- do you remember the P waves and the something else waves that he gave us a little tutorial about? Do you remember that bit of evidence?

LETBY: Yes.

JOHNSON: He said the trace of the heart was normal.

LETBY: I can't comment on that. That's not something that nursing staff could interpret.

JOHNSON: Have you ever seen an arrhythmia in a neonate?

LETBY: Um... No, I don't think so, no.

JOHNSON: Dr Jayaram said that [Baby A]'s heart was pumping normally; do you disagree with that?

LETBY: That's his opinion. I don't feel I can assess that.

JOHNSON: You don't contradict it?

LETBY: No.

JOHNSON: Do you say that Dr Jayaram is lying? Sorry, I shouldn't have asked that. It was put to him.

Dr Marnerides found an air bubble in [Baby A]'s brain and in his lung. Did you inject [Baby A] with air?

LETBY: No.

JOHNSON: Dr Arthurs found air bubbles in his great vessels; do you remember that?

LETBY: Yes.

JOHNSON: Just like [Baby D] and [Baby O].

LETBY: Yes.

JOHNSON: That's because you injected him with air, isn't it?

LETBY: No.

JOHNSON: After this, did you want to get straight back into nursery 1?

LETBY: Yes.

JOHNSON: Why?

LETBY: From my experience at Liverpool Women's, you are actively encouraged that if you've lost a baby in a certain cot space, that you go straight back into that nursery and care for other babies so that you sort of move on from that first experience and it doesn't almost prevent you from going back into the intensive care environment.

JOHNSON: Did you tell the truth to the police?

LETBY: In what respect? Yes, I have told the truth.

JOHNSON: Can we just go to your second interview, please, right at the end? It's [Initials of Baby A] 27. Can you see that about halfway down the text on the page, so that's about three or four inches from the top of the page, you're asked about the dangers of air going into the circulation?

LETBY: Yes.

JOHNSON: And you're asked about what the dangers are and you said:

"I don't know what it would cause."

Is that true?

LETBY: Yes, I don't know the pathological reasons that it would -- I know ultimately it would end in death, yes. Anything between that, I don't know.

JOHNSON: And the question was what, "What are the dangers"?

You have just told us death. Why didn't you give that answer?

LETBY: I can't answer that.

JOHNSON: Were you playing daft?

LETBY: No. It's something that every nurse would know.

JOHNSON: Oh yes, absolutely.

LETBY: Yes.

JOHNSON: So why didn't you say so?

LETBY: I've also said that I don't know exactly how it would affect a baby and that is true. I know the ultimate -- in anything that you do, the ultimate serious outcome could be death. What that would look at -- what that would appear as in symptoms of the baby, I don't know.

MR JOHNSON: I'm going to move on to [Baby B] next.

My Lord, we've done about an hour, actually.

MR JUSTICE GOSS: I was thinking that because it's a shorter total period after the break and we have to have an hour and 10 minutes, so we'd resume at 2 o'clock if we broke off now, so that seems to be a sensible place to break.

A bit earlier than normal, but we'll restart at 2 o'clock.

You'll remember that yesterday I reassured you that letters would be available for your employers and I was going to give you the best estimate I could in relation to how long you may still be engaged upon this trial.

I've already said that ultimately when it gets to your deliberations that's a matter entirely within your hands, but the letters that you're going to receive are going to warn your employers that potentially, and there are all sorts of unknowns that we don't know about yet, but potentially you may be here until around the end of July. I've given it what I call the worst-case scenario. Not a very appropriate expression, but the longest possible time foreseeably you could be here.

All right? If you work on that basis, but I anticipate it may be before then. That's what the letters to your employers will say, which I hope provides you with the necessary paperwork.

If not, raise it with the usher. Thank you very much indeed. 2 o'clock then.

(In the absence of the jury)

MR JUSTICE GOSS: Mr Johnson, are you all right?

MR JOHNSON: Yes, I'm fine.

MR JUSTICE GOSS: I'm conscious that obviously you're suffering from some sort of infection and I just saw you then... All right. If you genuinely feel so unwell that you can't --

MR JOHNSON: I'm fine.

MR JUSTICE GOSS: I thought you would raise it. All right.

Thank you very much.

(12.54 pm)

(The short adjournment)

(2.00 pm)

(In the presence of the jury)

MR JOHNSON: Is there anything that you said this morning that you would like to revise or amend?

LETBY: No.

JOHNSON: There's one thing that I overlooked, which I'd like to deal with. Would you go to the defence statement that I gave you yesterday, please? It was a loose -- that's it. Thank you.

Could you go to page 6? Paragraph 39. Would you read it out to the jury, please?

LETBY: "There was some blotchiness on [Baby A]'s body but no one discussed it with me and I don't remember it being mentioned amongst the nurses."

JOHNSON: Why didn't you put mottling?

LETBY: I think they're interchangeable.

JOHNSON: We move on to [Baby B], please. Before you were interviewed for the first time on 4 July 2018, did you have any memory, independent memory, of [Baby B]?

LETBY: Very little.

JOHNSON: Is it your case that staffing levels contributed to [Baby B]'s collapse?

LETBY: No, I don't know what caused her collapse.

JOHNSON: Is it your case that medical competence contributed?

LETBY: Again, I don't know why she collapsed.

JOHNSON: Have you identified any mistakes that were made with her treatment?

LETBY: No.

JOHNSON: Do you remember the devastation that [Baby A]'s death caused his family?

LETBY: Yes.

JOHNSON: Do you remember [Baby B]'s and [Baby A]'s dad literally being prostrate on the floor following [Baby A]'s death?

LETBY: What does that mean, sorry?

JOHNSON: On the floor, lying on the floor.

LETBY: I don't recall anyone lying on the floor, no.

JOHNSON: Well, can we go to [Baby B]'s sequence of events, please, to tile 132.

This is your message. Would you read it out, please?

LETBY: "Oh don't feel like that, I'm sorry you had to end your shift like that. I've said to [Nurse A] that I can't look after [Baby B] because I just don't know how I'm going to feel seeing parents. Dad was on the floor crying, saying, please don't take our baby away, when I took him to the mortuary. It's just heartbreaking. Glad the photos are nice."

JOHNSON: You don't remember that?

LETBY: No.

JOHNSON: Were you keen to give your colleagues the impression that you didn't want to take the responsibility for [Baby B]?

LETBY: It wasn't that I didn't take responsibility, it was just that I felt she'd be better looked after by somebody else.

JOHNSON: But didn't you feel that getting back into nursery 1 was the best thing to do, to get back on the horse like they do in Liverpool?

LETBY: Yes, but not with a baby and a family that we've dealt with before that have just lost their child.

JOHNSON: But you did migrate back to nursery 1, didn't you, despite that?

LETBY: We cover all of the nurseries.

JOHNSON: Yes. Would you answer the question, please? You did migrate back to nursery 1?

LETBY: At which point?

JOHNSON: Did you or didn't you?

LETBY: I will have been in nursery 1 at some point, yes.

JOHNSON: We'll get to the point.

Do you accept that [Baby B] did well on the day shift of 9 June, the day after her brother had died?

LETBY: I have no memory. I was not allocated for [Baby B], so I don't recall the handover.

JOHNSON: Right. Well, can we look at the interviews, please.

The 12th page of the [Baby B] interviews. What is your second answer on that page, please?

LETBY: "There hadn't been any undue concerns expressed."

JOHNSON: Yes, and that's about the clinical position of [Baby B], isn't it?

LETBY: Yes.

JOHNSON: So do you accept that [Baby B] did well on the day shift of 9 June?

LETBY: Yes.

JOHNSON: We received evidence that she was off CPAP that day, having skin-to-skin; do you remember that?

LETBY: Yes.

JOHNSON: If anybody wants to make a note, that's tiles 76 and 78.

That she was rooting for milk, we were told by Joanne Williams; do you remember that?

LETBY: No, but I accept that.

JOHNSON: Let's look. Tile 84. I'm not suggesting it's a memory test. This is Joanne Williams' note. Just scroll down, please. I think the -- that's fine. Can you see on about the sixth line or so?

LETBY: Yes:

"Rooting, looking for feeds."

JOHNSON: Yes. That is a positive picture, isn't it?

LETBY: Yes.

JOHNSON: So the signs were good; do you accept that?

LETBY: Yes, on looking at the notes, yes.

JOHNSON: Do you recall the evidence from [Baby B]'s family that in effect they stood guard over her following the death of [Baby A]?

LETBY: I know that they were very much present on the unit, yes, and allowances were made for that, yes.

JOHNSON: But they left, according to the agreed evidence, shortly after 8 pm; do you remember that?

LETBY: I don't remember, but I accept if that's agreed, yes.

JOHNSON: Yes. It was evidence that was read to the jury by agreement.

LETBY: Okay.

JOHNSON: I'd like to go to the staff presence at tile 145, please, with Mr Murphy's assistance.

There we see the population distribution and the corresponding responsible nursing staff, don't we?

LETBY: I can see the nursing staff, yes, not the --

JOHNSON: Sorry, yes. It's because I've got a sheet of paper that's got both on. If we move on to the next page, Mr Murphy. It's my fault, for which I apologise.

We can see that in nursery 4, Lisa Walker was working?

LETBY: Yes.

JOHNSON: And she was working the day before, wasn't she?

LETBY: I don't recall that myself.

JOHNSON: Okay. It's a matter of record.

LETBY: Okay.

JOHNSON: Your friend, [Nurse A], was working?

LETBY: Yes.

JOHNSON: So she is the second commonality in terms of the staff. Your friend, Mary Griffith, was working as well, wasn't she?

LETBY: Yes.

JOHNSON: And she was your friend, wasn't she?

LETBY: Yes.

JOHNSON: And she too had been working when [Baby A] had died the night before?

LETBY: No. Mary Griffith wasn't there with [Baby A].

JOHNSON: Wasn't she?

LETBY: No.

JOHNSON: All right. It may be my mistake. I'll check that and either apologise or make sure that we get it right.

Kathryn Percival-Ward was the shift leader?

LETBY: Yes.

JOHNSON: She had a single child in nursery 4?

LETBY: Yes.

JOHNSON: Did you get on with her?

LETBY: Yes.

JOHNSON: Does she bear you a grudge?

LETBY: No, I got on well with all of the nursing colleagues.

JOHNSON: She hadn't been working the previous night though, had she?

LETBY: No.

JOHNSON: Do you remember the evidence that she gave to the jury on 21 March this year, in the case of [Baby P], when she told the jury that you had said to her that working in nurseries 3 and 4 was boring?

LETBY: Yes, I remember that.

JOHNSON: And that you would, to use her word, migrate back to nursery 1 when things happened?

LETBY: Yes.

JOHNSON: And that is true, isn't it, you did migrate back to nursery 1?

LETBY: Yes, in response to things that were happening, which any other member of staff would be expected to do as well.

JOHNSON: Well, we will see.

You had two children in nursery 3; is that right?

LETBY: Yes.

JOHNSON: And you didn't like being there, did you?

LETBY: That's not accurate at all, no.

JOHNSON: You were bored?

LETBY: No, I've never been bored at work, I've never described my work as boring.

JOHNSON: Well, let's deal with what happened prior to [Baby B]'s collapse just after midnight. Again, if Mr Murphy can help, and if you wouldn't mind yourself, Ms Letby, looking at the neonatal review. It's the first bundle of neonatal reviews, which is in the white file.

Your babies, as we have just established, were HM and EB.

LETBY: Is it not HT?

JOHNSON: HM.

LETBY: Sorry, on there it's HT.

JOHNSON: It's HT on there, is it?

LETBY: Yes, it is.

JOHNSON: Sorry.

LETBY: But yes, it's that baby, yes.

JOHNSON: Right. The difficulty then is that the same child appears as HM in the neonatal review. I've got the full names here and I can understand why that mistake has been made, but it's the same child. So let's --

LETBY: So HT is in nursery 3 and on transitional care with Cheryl Taylor? Is it two separate babies?

JOHNSON: Sorry, yes, there's an HT and an HM.

LETBY: They're both HT on here.

JOHNSON: Okay. Well, the one on transitional care is HT and the one in nursery 3, who was your baby, is HM.

LETBY: Right.

JOHNSON: Okay?

LETBY: Okay.

JOHNSON: I'll just show Mr Myers what I've got here and confirm for...

(Pause)

We're agreed, HM in the neonatal review is in nursery 3.

LETBY: Okay.

JOHNSON: So as a matter of fact, just so that the jury understands, and you your legal team have had access to all this material unredacted, haven't you, so that the unanonymised versions of the documents you have seen?

LETBY: Of the layout, like this?

JOHNSON: Yes, both -- this layout with the full names and the documents that underlie the neonatal review, so the medical records --

LETBY: Yes.

JOHNSON: -- of these other children have been served on you and your legal team in an unedited form so that you see everything.

LETBY: Yes.

JOHNSON: Yes. All right. So let's just deal with what you were doing at the beginning of the shift, if we may, from the neonatal review.

We see in lines 1, 3, 6, 9 and 10 that you were dealing with HM and child EB.

LETBY: Yes.

JOHNSON: They're both your children, in effect?

LETBY: Yes.

JOHNSON: You're the designated nurse. That continues down towards the bottom of the page at 22.00 hours at lines 25, 26, 27 and 30. Are we agreed so far?

LETBY: Yes.

JOHNSON: Over the page, on page 2, we have four lines, 38 to 42, where you're dealing with HM at or about 23.00 hours?

LETBY: Yes.

JOHNSON: Then there's the nasal prongs incident, if I can call it that, at line 47 at midnight.

LETBY: Yes.

JOHNSON: Then over the page, line 70, we have the event for [Baby B] when the prosecution say she collapsed.

LETBY: Yes.

JOHNSON: I suggested to you a moment ago that you were bored and you did not accept that. So if we go now, please, to the sequence, Mr Murphy, I'd be very grateful, starting with tile 151.

That's you texting Yvonne Griffiths, isn't it?

LETBY: Yes.

JOHNSON: And this shows, doesn't it, as a matter of fact, that as I suggested to you earlier, the parents of [Baby A] and [Baby B] were still on the unit at this time?

LETBY: Yes.

JOHNSON: At tile 165, please, it shows that between 20.41 and 21.00 hours you were texting your friend, Minna Lappalainen.

LETBY: Yes.

JOHNSON: And as we've just seen, that's just after you had been dealing with EB and HM in nursery 3?

LETBY: Okay.

JOHNSON: And this is why I'm suggesting you're bored because you're engaging in chit-chat with your friend.

LETBY: No, that's common practice, we all used to do that on the unit. That's not unique to me.

JOHNSON: Tile 169. More messages to and from Minna in a ten-minute period between 21.00 and 21.10.

LETBY: Yes.

JOHNSON: Then Jennifer Jones-Key in the next tile:

"Manic. Hope you're okay."

And you respond immediately:

"Manic?"

And then you at 172:

"Mel took it hard, we spoke earlier. Thank you, we're a good team. Say hi back."

And those messages continued with both [Nurse E] and Jennifer Jones-Key, and indeed Yvonne Griffiths, through the tiles up to and including tile 181 at 21.32.

If you could flick through them very quickly, please, Mr Murphy, to 181.

(Pause)

You are bored?

LETBY: No, all members of staff use their phone on that unit.

And this is me replying to our ward manager, who has text me whilst I'm on shift. It was accepted that we used our phones on the unit.

JOHNSON: We can take it then that staffing levels weren't an issue on this shift; is that right?

LETBY: I can't comment for the whole of the shift. Obviously at this point my babies have been cared for.

JOHNSON: What do you mean, you can't comment for the whole of the shift?

LETBY: I'm not responsible for the other babies so I can't say what care needs they are having and receiving at this time.

JOHNSON: Well, are you suggesting that there was some deficiency in staffing levels?

LETBY: Not at this moment. I can't see what was going on with the other babies.

JOHNSON: Let's continue with your messages, please. If we could go to 186 next, please. You asking Jennifer Jones-Key whether Yvonne Griffiths was in; is that right?

LETBY: Yes.

JOHNSON: 187, she replies. 188, her clarification. 189, your reply. 190, her reply. 194, another one from you.

195, 196, 197. A block of social messages for 27 minutes between 22.01 and 22.28.

LETBY: Yes.

JOHNSON: Can we go back to the neonatal review chart, please?

Page 2.

In the middle of that block of messages, line 30, you signing for a prescription for HM.

LETBY: Yes.

JOHNSON: You didn't use your phone in clinical areas?

LETBY: No, I didn't use my phone in the nurseries, no, that's right.

JOHNSON: At tile 208, please -- sorry, 202. Could we have tile 202, Mr Murphy? My mistake.

A further block of social messaging between you and someone called Charlotte James. Who is Charlotte James?

LETBY: I lived with Charlotte James in the staff accommodation at Ash House.

JOHNSON: So is she another nurse or employee of the hospital?

LETBY: Yes.

JOHNSON: Were you bored?

LETBY: No.

JOHNSON: As a matter of fact, do you spend a lot of time texting when you're in either nursery 3 or nursery 4?

LETBY: I text regardless of where I am on shift. It's a common -
-

JOHNSON: So even if you've got an ITU baby?

LETBY: At times when they're not needing care, yes, and I think everybody would say the same thing if they were honest.

JOHNSON: If we go back to the neonatal review and just pick up where I left off, please, at page 3 of 7. We see that at 23.02 and 23.03, that block of five entries in respect of which there's probably a degree of similarity in the sense that the prescription and the update is the same event, isn't it?

LETBY: Yes.

JOHNSON: So there's double counting, if you like. We see that's what you were doing with your friend, [Nurse A], at that time; is that right?

LETBY: Yes, I was on the computer at this time, yes.

JOHNSON: We know from the evidence, and it's tile 47, that [Baby B] desaturated at about midnight; is that right?

LETBY: Yes.

JOHNSON: And if Mr Murphy wouldn't mind, by going back to the sequence of events to tile 209, we have that event represented by that tile; is that right?

LETBY: Yes.

JOHNSON: Immediately afterwards in a succeeding tile, 210, we see [Nurse A]'s notes relating to it.

LETBY: Yes.

JOHNSON: That's when you migrated back into nursery 1, wasn't it?

LETBY: I was working in nursery 1 at points, yes, with medications.

JOHNSON: After this time you were in nursery 1, I'm going to suggest.

LETBY: Yes.

JOHNSON: So I'm sorry to have us jumping around. It may be easier just to stick with the paper documents at the moment rather than having things coming and going on the screens. But if we go back to the neonatal review, page 3 of 7, lines 62 and 63, we see you involved, first of all, with [Baby B] immediately after her desaturation.

LETBY: Yes.

JOHNSON: Line 62. What was going on at that point?

LETBY: So from this here, Babiven and lipid was being given.

JOHNSON: Yes. This was a new bag, wasn't it?

LETBY: I can't say from looking at this on its own.

JOHNSON: Okay.

LETBY: It may have been a continuing 48-hour bag, I can't say from this.

JOHNSON: Let's go, having said we wouldn't do this, back to the sequence of events then, please, Mr Murphy. Tile 213.

You tell us whether it was a new bag. If we scroll down, my references don't equal what's on the screen at the moment, but we'll work our way round it. This is a prescription for lipid; is that right?

LETBY: No, this is the TPN and lipid together.

JOHNSON: Yes, right, thank you. Whose signature appears on it?

LETBY: Myself.

JOHNSON: Who else's?

LETBY: No one's.

JOHNSON: Shouldn't there be another one on there?

LETBY: Yes. This isn't the form that we both sign, there's an additional form.

JOHNSON: But that's not the question. Shouldn't someone else's signature be on this document?

LETBY: Um... Potentially on this document, but as long as it's on the other sheet that would be acceptable.

JOHNSON: Potentially. Should another person's signature be on this documents?

LETBY: Yes, ideally, yes.

JOHNSON: Thank you. Here's the other document. What do we see here on day 3, 9/6/15?

LETBY: It's got myself and [Nurse A] starting a new bag of TPN and lipid at 00.05.

JOHNSON: So you have migrated back to nursery 1?

LETBY: All medications have to be checked by two nurses so inevitably it would have to be another member of staff that was assisting [Nurse A].

JOHNSON: All right. Let's go to the next tile then, please.

Let's concentrate on what's going on at midnight. 24.10 is the way it appears there; is that right?

LETBY: I can't see that clearly.

JOHNSON: We'll make it bigger. 24.10? About the time the Babiven bag goes up?

LETBY: Potentially. That's not my writing, but it looks either 00 or 10, yes.

JOHNSON: Okay. Then if we scroll down, whose writing is that number there?

LETBY: I can't say.

JOHNSON: Whose writing appears in the next column?

LETBY: Myself. It looks similar, yes.

JOHNSON: 45 is you, isn't it?

LETBY: The what, sorry?

JOHNSON: The 45 in the 24.10 column?

LETBY: Oh, potentially, yes.

JOHNSON: "Potentially, yes", does that mean yes?

LETBY: I can't be definitive because it's not my writing at the top.

JOHNSON: You had migrated back into nursery 1, hadn't you?

LETBY: That's an expected course: whoever's working in nursery 1 is going to need assistance from another nurse.

JOHNSON: What, to do the observations?

LETBY: I'm not saying that I've done the observations, but potentially, yes, if [Nurse A] was busy doing something else then we would support each other, yes.

JOHNSON: Let's look at the blood gas a few minutes later at tile 215, 00.16. Whose writing is that?

LETBY: That's mine.

JOHNSON: You had migrated back into nursery 1, hadn't you?

LETBY: A blood gas needs two nurses, so yes, two nurses would need to be there and, yes, I was there.

JOHNSON: Well, you say that, but as a matter of fact, as we have gone through all the blood gas documents in this case, it's almost always the designated nurse that makes the entry in the chart, isn't it?

LETBY: No, I don't agree with that.

JOHNSON: You don't agree?

LETBY: No.

JOHNSON: Well, I'm not going to go through all of them now, you'll be glad to know.

Now, when you were being asked questions about this a while ago now, you said that [Nurse A] had remained with [Baby B] at this stage; do you remember that?

LETBY: Yes, she was in the nursery with her, yes.

JOHNSON: No, you said she actually was with her. Do you remember saying that?

LETBY: No.

JOHNSON: You don't remember that?

LETBY: No. Was that a police interview?

JOHNSON: No, no, that's when Mr Myers was asking you questions.

LETBY: I know [Nurse A] was with her, she realigned the prongs, and then we put the TPN bag up, so yes, she was with her and she carried out the blood gas.

JOHNSON: Do you remember [Nurse A] telling the jury that in fact she was diverted by preparing medication for another child in nursery 1?

LETBY: Yes, we were both preparing that medication.

JOHNSON: And that's why you were doing the observations on [Baby B], isn't it, because she was actually doing --

LETBY: I can't say that. I was checking medication with [Nurse A].

JOHNSON: She was the sterile nurse.

LETBY: Yes.

JOHNSON: She was doing the medication and you were doing the other jobs.

LETBY: No, I would have to be by her side, watching everything that she was doing. That is the role of the second checker.

JOHNSON: So how did you do the blood gas at 00.16?

LETBY: I have not. I've run that through the machine for [Nurse A].

JOHNSON: But you've got to be with her while she's preparing the medication for the other child. How can you go to the blood gas room and be with her?

LETBY: What time is the medication for the other child.

JOHNSON: She told us that that's what -- when she gave evidence that is what she was doing at this time. She also said she was the other side of that little wall in the nursery.

LETBY: Yes, we were both there preparing medication, yes.

JOHNSON: So you do remember?

LETBY: I remember us both being in the nursery, yes.

JOHNSON: No, you remember being the other side of that little wall from [Baby B], do you?

LETBY: No, I remember us preparing medication that is where the medication is prescribed and drawn up.

JOHNSON: Do you remember telling the jury that it was [Nurse A] that had alerted you to [Baby B]'s collapse?

LETBY: Yes, I'm unsure which way round it was.

JOHNSON: Which way round what was?

LETBY: I'm unsure whether I spotted it first or whether [Nurse A] alerted me.

JOHNSON: Right. That's why you told the jury that she alerted you because you weren't sure which way round it was?

LETBY: My memory is she alerted me, but I can't say for definite now. That was my memory at the time.

JOHNSON: So she's wrong about that?

LETBY: Potentially, yes.

JOHNSON: Well, everything is "potentially yeah", but are you saying she's wrong?

LETBY: I can't sit here now and definitively say which way, no.

JOHNSON: You injected [Baby B] with air, didn't you?

LETBY: No, I didn't.

JOHNSON: Do you remember on Friday, 5 May, you were asked by your counsel about what happened to [Baby B] at this point?

LETBY: Yes.

JOHNSON: I'll remind you of what you said and ask you whether you want to change any of it. The question you were asked was:

"Question: Do you recall how [Baby B] was? When you say her colour had changed, could you describe that?

"Answer: She had become quite mottled and dark."

LETBY: Yes.

JOHNSON: Is that accurate?

LETBY: Yes.

JOHNSON: You were then asked:

"Question: All over her body or on particular parts of her body?

"Answer: All over from memory."

LETBY: Yes.

JOHNSON: You were then asked:

"Question: Can you describe that colour again?

"Answer: It was a dark, mottling colour."

LETBY: Yes.

JOHNSON: "And you saw that", you were asked, and you said yes.

LETBY: Yes.

JOHNSON: You were then asked the question:

"Had you seen anything like that colouring before?"

LETBY: Yes.

JOHNSON: Do you remember what your answer was?

LETBY: No.

JOHNSON: "What do you mean?"

You were asked the question again:

"Had you seen that type of colouring on any baby before this point?"

And your answer, do you remember?

LETBY: No.

JOHNSON: "Yes, it was like general mottling that we do see on babies."

LETBY: Yes, and I agree with that.

JOHNSON: I know you do. Then you were asked the question:

"Question: Right. Was it unusual or was it not unusual in your opinion?

"Answer: It was not unusual, but obviously we were concerned for [Baby B] because of [Baby A]'s decline the night before."

LETBY: Yes.

JOHNSON: And so the essence -- I am going to suggest to you that the essence of what you were saying to the court on Friday, 5 May was that [Baby B] had what I will describe as common or garden mottling.

LETBY: I would say her mottling was more pronounced maybe than general, but yes, it was mottling.

JOHNSON: Have you always said that?

LETBY: I don't recall.

JOHNSON: You don't recall? Why would you say anything different?

LETBY: That's how I remember it now, yes.

JOHNSON: Okay. If we go to your interview, please, so back to file number 1. It's the first of the [Baby B] interviews, pages 7 to 8.

Right at the bottom of the page, page [Initials of Baby B] 7, please, you're asked a question about five or six lines up saying:

"Question: Okay, and that mottling, as you've described it, is it something you witnessed in [Baby B] yourself?

"Answer: Yes.

"Question: You did? When was that?

"Answer: I don't recall at what point.

"Question: Right. And the circumstances around you seeing the mottling? Who was with you at the time?

"Answer: I think it was [Nurse A].

"Question: Can you describe that mottling? Is it similar to that with [Baby A], different?

"Answer: I think she was more mottled as opposed to -- [Baby A] was paler. [Baby A] was more pale centrally, mottled peripherally, and I think from what I recall [Baby B] was more mottled and that extended over more of her body rather than just her limbs.

"Question: So was it a different colour?

"Answer: From what I remember, it was darker, just darker than [Baby A]. So [Baby A] was pale whereas [Baby B] was more mottled, which is a sort of purply-red."

LETBY: Yes.

JOHNSON: Is that true?

LETBY: Yes.

JOHNSON: Right, okay. "Rash appearance" is the phrase you then use, "rash appearance". This isn't mottling, this isn't common or garden mottling you're describing, is it?

LETBY: Yes. It's more pronounced mottling, it's darker mottling, but it is mottling.

JOHNSON: Why did you use the word "rash appearance"?

LETBY: I can't recall now.

JOHNSON: What does it mean to you now? These are your words.

What do they mean?

LETBY: I don't know, I would take "rash appearance" to mean that it was all over the body.

JOHNSON: The next question:

"Any particular shapes to that?"

And your answer, please?

LETBY: "Well, usually mottling is sort of a patchy round appearance."

JOHNSON: Yes. Are you saying this was normal?

LETBY: No, mottling is not normal, but it is something that we see.

JOHNSON: Okay. In the normal parameters of mottling, are you saying this was normal?

LETBY: No, it was more pronounced than just general mottling.

JOHNSON: If it was -- do you think it was remarkable?

LETBY: I can't recall specifically. I know once we saw it, it came very quickly, and in view of [Baby A]'s deterioration, everybody acted very quickly.

JOHNSON: You say:

"Once we saw it, it came very quickly."

Were you with [Baby B] before it appeared?

LETBY: No.

JOHNSON: Why did you say:

"Once we saw it, it came very quickly"?

LETBY: That's just -- I've re-phrased that wrong, sorry.

JOHNSON: That's all right. Well, it's all right. You just say what you mean. If this was all fairly normal, why did the doctor ask you to go and get a camera?

LETBY: In view of what had happened with [Baby A] the day before, we did not want to take any chances and wanted to document everything that was happening.

JOHNSON: Let's look at what other people said about this and see whether you agree or disagree with them. Okay? I'm going to start on this occasion with [Mother of Babies A & B], who is [Baby A] and [Baby B]'s mum:

"It was a very similar situation to [Baby A]: [Baby B]'s heart and oxygen saturation levels had fallen rapidly. The female consultant went on to describe seeing blotches/mottling across her skin, hand and feet. She asked our permission to take pictures of the mottling and she, that is the consultant, stated she had never seen this before."

This is [Dr B], one of the Gang of Four:

"I remember being surprised that this was something she had never witnessed before. I imagined a consultant of neonatal care would have seen everything. By the time the staff had organised a camera it had disappeared."

Do you accept what [Mother of Babies A & B] says?

LETBY: I accept that there was mottling, yes.

JOHNSON: Do you accept that [Dr B] was saying at the time, "I have never seen anything like this before"?

LETBY: I can't recall that conversation, no.

JOHNSON: But you have agreed the evidence.

LETBY: Sorry, I don't understand.

JOHNSON: [Mother of Babies A & B] didn't give evidence because what she says in her witness statement is agreed.

LETBY: Yes.

JOHNSON: So it is agreed that [Dr B] said, "I have never witnessed this before".

LETBY: Yes, but it was not said to me.

JOHNSON: No. You agree it was said?

LETBY: Yes.

JOHNSON: And do you remember it being said?

LETBY: No.

JOHNSON: Why are you prepared to agree this then when you're not prepared to agree other witnesses' accounts where they don't accord with -- simply on the basis that you didn't see what they say they saw?

LETBY: Can you rephrase that, please?

JOHNSON: Yes, it wasn't very well put, I'm sorry.

When I was asking about [Baby A], you would not accept the descriptions given by some of the witnesses because you say that was not what you saw.

LETBY: Yes.

JOHNSON: Here we have something that you say you did not hear that you are prepared to accept; is there a reason for that?

LETBY: I'm relying on that person being accurate. I wasn't there for that conversation so I can't say whether that was said or not.

JOHNSON: Did you go and get the camera?

LETBY: Yes.

JOHNSON: Were you keen to get a picture of what it was that was so unusual?

LETBY: No, I was asked to go and get the camera, which I did immediately, that was what the doctors requested, and by the time I came back her skin was normalised.

JOHNSON: Do you accept that this was mottling that is not normal mottling?

LETBY: It was more pronounced mottling, yes, and in view of what had happened with [Baby A] all of the staff were very keen to act quickly on any change with her in case it was anything that was related to [Baby A].

JOHNSON: Do you remember Dr Lambie?

LETBY: Yes.

JOHNSON: She made a note, that's at tile 243 if anybody wants to keep a note, but I'll remind you of what she actually said in evidence:

"I do remember she was being resuscitated when I arrived. She was a very dusky, pale grey colour."

Is that accurate?

LETBY: I can't say at that point. I don't know if I was there at that point.

JOHNSON: "As we were helping her, she then -- she was then developing widespread blotches."

Do you accept that is accurate?

LETBY: Not from my memory, no. It was mottling.

JOHNSON: This wasn't disputed in cross-examination. You understand that? This wasn't challenged.

LETBY: Yes, but I wasn't there at that point, potentially I was getting the camera.

JOHNSON: Well, do you accept it?

LETBY: If that's what she says she saw, then yes.

JOHNSON: Okay. I accidentally didn't read the full sentence there, so I'll read the full sentence:

"As we were helping her, she was then developing -- widespread blotches is probably the best way of describing it, patches of a purply/red colour."

Do you accept that?

LETBY: I don't recall ever seeing that, but I accept if that's her -- if that's what she thinks she saw, then yes.

JOHNSON: "They would flush up, last about a few seconds, 10 seconds, and then disappear and then appear elsewhere."

Do you accept that?

LETBY: No. I did not see anything like that and no conversation was ever had about that.

JOHNSON: Therefore, do you say that that is untrue?

LETBY: I can't comment for them, but it's not something that I saw or was aware of, no.

JOHNSON: No, but there's a difference between accepting something you have not seen and disputing something you have not seen. Do you accept the evidence?

MR MYERS: My Lord, we're getting, again, close to the same territory. We just asked Ms Letby if she says it is untrue, which is another way of saying, for example, "Is that a lie?" She's been very clear as to what she says she saw and why she can't say any more about it.

To then go on beyond that -- there may be matters to be made in due course, but if the witness has said she didn't see that and can't say, that surely is the end of the matter. And asking her, "Is it untrue", takes us back to where we were earlier on.

MR JOHNSON: I'll move on.

MR JUSTICE GOSS: Perhaps I could just ask a question: is there anything that leads you to conclude that she may have been mistaken in what she was seeing?

LETBY: What...?

MR JUSTICE GOSS: Anything you were aware of that you can now remember that could have been mistaken for what she has described?

LETBY: No. I just recall seeing mottling.

MR JUSTICE GOSS: The sort of purply-red mottling?

LETBY: Yes, that's mottling, yes. And I then went to get the camera.

MR JOHNSON: Do you remember, when being asked questions on your behalf, Dr Lambie said that she had never seen anything like this before?

LETBY: Yes.

JOHNSON: Your friend, [Nurse A], she said that what she saw was the same as [Baby A].

LETBY: I don't agree with that.

JOHNSON: What [Nurse A] said is:

"She suddenly looked very ill, she looked very like her brother had done the night before with the pale white, with this purple blotchy discolouration."

And she indicated over her chest:

"I just remember thinking, no, not again. I've never seen anything like that before and then to see his sister with the same appearance."

Remember that?

LETBY: Yes.

JOHNSON: That's the truth, isn't it, Lucy Letby?

LETBY: No, I do not agree with that. That is not what I saw.

JOHNSON: Finally we had [Dr B] saying:

"I have seen that there's a purple blotchiness to the right side, mid-abdomen..."

It's a bit like what you're describing with [Baby A], isn't it, one side of discolouration?

LETBY: It was paleness.

JOHNSON: But on one side?

LETBY: Yes.

JOHNSON: She said:

"... and right hand and the baby is pink and active."

Remember [Nurse A], of course, was considerably senior to you, wasn't she?

LETBY: Yes.

JOHNSON: You'd been looking after the sickest babies for 2 months.

LETBY: I don't understand, sorry.

JOHNSON: You'd been QIS trained for a couple of months, 2 months?

LETBY: Yes.

JOHNSON: Three months maybe, I don't know.

LETBY: Yes.

JOHNSON: She had a couple of decades of experience looking after the sickest babies?

LETBY: Yes.

JOHNSON: And yet, all of a sudden, once [Baby B] had had this episode, in effect you elbowed out [Nurse A] and took over the care of [Baby B], didn't you?

LETBY: No, I did not take over the care of [Baby B].

JOHNSON: We've already seen the observations at 1 o'clock. You carried those out, didn't you?

LETBY: Yes. [Nurse A] was supporting the parents at that point.

It's not unusual for nursing staff to assist each other with obs or fluids or medications. That's not her handing the care over to me.

JOHNSON: Let's look at tile 226, please. In the bottom left, at 01.00 hours, that's your writing, isn't it, and your initials at the bottom?

LETBY: Yes.

JOHNSON: "Intubated..."

It says 23.36, I think that's a mistake, isn't it, on the times?

LETBY: Yes.

JOHNSON: It was actually 01 or 00.36?

LETBY: Yes.

JOHNSON: 232, please. A blood gas record at 00.51. Your writing again?

LETBY: Yes.

JOHNSON: As it had been at 00.16?

LETBY: Yes.

JOHNSON: 238, please. 01.00 hours. Is that you making the entries at 01.00 hours?

LETBY: Yes.

JOHNSON: 241, 01.10. You and Mary Griffith signing for morphine for [Baby B]?

LETBY: Yes.

JOHNSON: 250 to 251. You and [Nurse A] co-signing for medications at 02.07?

LETBY: Yes.

JOHNSON: 253 to 256, more medications at 02.25?

LETBY: Yes.

JOHNSON: Were you suggesting to any of your colleagues that perhaps antiphospholipid syndrome was an explanation for the death of [Baby A] and the collapse of [Baby B]?

LETBY: No, because I wasn't aware that -- I didn't have any details about what antiphospholipid syndrome was. I'd never heard of it before.

JOHNSON: Let's just go back to the [Baby A] sequence if we can, please, Mr Murphy. Sorry to jump around. Can we have tile 337, please?

Querying a clotting problem. That's what antiphospholipid syndrome is, isn't it?

LETBY: Yes, I know that now, yes.

JOHNSON: You knew that then because you were sending the text.

LETBY: No, that was the discussion, they were querying a clotting problem. I don't think the word antiphospholipid syndrome was used at that point.

JOHNSON: Can we go to -- I'll just check something. Back to the interview, please, at [Initials of Baby B] 7, a reference I missed out earlier. Almost exactly halfway down the page, what did you say in answer to the question:

"... or any observations that you had with [Baby B] that caused you concern at all on that shift?"

LETBY: "I do remember that she had some mottling that looked a little bit similar to [Baby A]'s appearance the day before."

JOHNSON: You told the jury that all you saw on [Baby A] was whiteness.

LETBY: Yes.

JOHNSON: So how did her mottling look similar to what [Baby A] (overspeaking) --

LETBY: I have then gone on to describe it in more detail.

JOHNSON: Yes, but it's not the description --

LETBY: (Overspeaking) they were different.

JOHNSON: It's not the description I'm interested in, I'm interested in why you were linking or drawing a similarity between what you had seen on [Baby A] and what you had seen on [Baby B].

LETBY: I can't say now. I don't know.

JOHNSON: Well, I'll offer you a reason: because they were the same.

LETBY: No, I don't agree with that.

JOHNSON: Like your friend, [Nurse A], said:

"Oh no, not again."

LETBY: She was referring to a sick baby, she didn't want something happening again. She wasn't referring to the rash.

JOHNSON: No, no, she was referring to the rash, Lucy Letby.

LETBY: Okay.

JOHNSON: That's precisely what she was referring to.

LETBY: Okay. I don't agree with that.

JOHNSON: I know you don't agree. But you're also not going to be able to rewrite what she said, are you?

LETBY: No.

JOHNSON: No. Do you accept that all the people who saw the skin discolouration say that they hadn't seen this sort of thing before?

LETBY: I have to accept what they say, yes.

JOHNSON: Do you accept that Professor Kinsey has excluded a blood disorder as a cause for what was seen?

LETBY: Yes.

JOHNSON: Do you accept that Dr Arthurs saw the bubbles in [Baby A]'s case?

LETBY: Yes.

JOHNSON: And that Dr Marnerides found them in the histology also in [Baby A]'s case?

LETBY: Yes.

JOHNSON: Do you accept that air was put into the IV lines of both children?

LETBY: No.

JOHNSON: Or either of them?

LETBY: No.

JOHNSON: Do you accept that you had the opportunity to have access to the IV lines of both children, just before they collapsed?

LETBY: Yes, but I didn't access the lines.

MR JOHNSON: My Lord, that's probably a convenient point.

MR JUSTICE GOSS: Yes, certainly. We'll have about 15 minutes then, please.

(In the absence of the jury)

MR MYERS: My Lord, there is a matter that arises that I'd like to raise now because I see the way in which certain topics are being dealt with by the prosecution and if the concern I raise is correct, I'd rather raise it now than wait. It's to deal with the form of questioning in cross-examination. I'm assisted by the commentary in Archbold that deals with this, which is at section 8-258 if your Lordship wishes to look at that --

MR JUSTICE GOSS: I think I know what you're going to say about putting -- inviting a witness to comment on the evidence of another witness.

MR MYERS: Yes, because what's actually happening is, under the guise of reminding the witness or the jury of another witness's evidence, what we're actually having is rehearsal of their evidence and then the defendant really being invited to comment upon it and extended questioning upon that, which, in my submission, goes well beyond what's acceptable with it.

We do object to that. It's one thing for a witness to be prompted and, in a long trial, for the jury to be assisted by having attention drawn to something that was said. And then asking what the witness in this case, the defendant, says in response. But to then persistently go beyond that and asking, "Well, what do you say they say that for", or, "Are they telling the truth", or, "Are all these people wrong", questioning like that. We're quite plainly going into territory that is a matter of argument, really, and going well beyond the form of questioning for cross-examination.

MR JUSTICE GOSS: Well, I did try to give Mr Johnson a steer in relation to what I consider might be an appropriate question in relation to this. Mr Johnson, do you wish to say anything about this?

MR JOHNSON: Well, I don't want to get involved in a protracted submission about it, but there's a world of a difference between inviting a defendant's comment on evidence that has been given, particularly evidence that is agreed or not disputed, and it's a standard part of any criminal trials that defendants are reminded of what witnesses have said and asked in effect how their account lives with that.

MR JUSTICE GOSS: It's that last part that is the controversial part. That's why I sought to try and go as far as I think is proper to say (overspeaking) "Is there anything that you can

recall that would undermine or contradict what the witness is saying?" That avoids the difficulty.

Then I think -- I don't know whether you've got other questions in relation to [Baby B] or not.

MR JOHNSON: There's just the correction, which was helpfully passed to me, which is that Mary Griffith was working both shifts.

MR JUSTICE GOSS: That was exactly what I was going to say to you. I immediately looked at the neonatal review and saw her name appearing all through the shift. Sometimes as Griffith and sometimes Griffiths. That has been a common feature of the trial. It is Mary Griffith, but she's sometimes referred to as Griffiths. There we are, that's it.

If you need -- if you can't be ready for quarter past, will you let me know? I don't know whether that will be long enough or not. Thank you very much.

(3.02 pm)

(A short break)

(3.15 pm)

(In the presence of the jury)

MR JOHNSON: If you wouldn't mind putting those interviews to one side, please. On the issue of the presence or absence of Mary Griffith on both [Surname of Babies A & B] shifts, could you just turn back in that neonatal review to the review for [Baby A], please?

If you go to page 3 of 7, you'll see that Mary Griffith's name appears quite a lot on that particular shift, and indeed on other pages, so it seems that for once I was right and she was a person in common on both shifts.

LETBY: I think I was just referring that she wasn't directly involved with [Baby A].

JOHNSON: It's all right.

LETBY: I accept that she's at work, yes.

JOHNSON: All right. We're not going to fall out over something trivial like that.

Now, we're moving on to [Baby C], born on 10 June 2015 and died at about 5 am on 14 June, although the event for which we allege you to be responsible that caused his death happened just before midnight on 13 June and, more particularly, at about 11.15 that night, 23.15.

I'd like to start with your defence statement on this one, please. My Lord, this is a document that the jury don't have at the moment, but they will have a copy in due course. As always, with these documents, they need to be edited by agreement.

I think it's on the desk in front of you. Would you please just turn to it? I want to deal with the section of the document that deals specifically with [Baby C]'s case, albeit there are other parts of the document that have a bearing on it and we will deal with in due course. So page 7, please.

It starts with you saying that you did nothing to hurt him at paragraph 50; is that right?

LETBY: Yes.

JOHNSON: You say that on the night of the 13th to 14th June, you had the care of another child, JE, in nursery 3?

LETBY: Yes.

JOHNSON: At 52 you say:

"My recollection of events prior to the collapse of [Baby C] is vague, but I don't remember being in nursery 1 and I don't think Sophie Ellis is correct when she says I was."

LETBY: Yes.

JOHNSON: Paragraph 53:

"I think I heard the alarms going off but I didn't go straight to [Baby C]. I don't know whether I was walking past the nursery or how I came to go in there. It may have been I was called in because of the collapse, but at night-time sometimes we have to go into nursery 1 even if we're working in nursery 3, for example, to check equipment for the ITU spaces, but I don't recall being in there when [Baby C] collapsed."

(Pause)

I've just been asked to consider a specific point, my Lord. It won't take me long, but I'll just check it.

(Pause)

I can't find the immediate answer to the question I have just been asked and, rather than holding things up, I'll miss out a couple of paragraphs for now. We'll return to them later. We're not going to finish this today in any event.

Paragraph 56:

"I did have some contact with [Baby C]'s family, but not to the extent described by his mother."

As a matter of fact, just on that point, [Baby C]'s mother's evidence, [Mother of Baby C]'s evidence, was agreed, wasn't it? Do you remember it was read to the jury?

LETBY: Yes.

JOHNSON: "I didn't speak to [Baby C]'s mother asking if she wanted a priest to be called, nor was I the person who handed [Baby C] to his mother after we had stopped resuscitation.

I don't think I helped with the footprint or taking a lock of [Baby C]'s hair. I do not recall asking if the parents wanted to put [Baby C] in a basket or any awkward conversation."

And again, that specific point, putting his body in a basket, is something that [Baby C]'s mother suggests you said and was agreed evidence, wasn't it?

LETBY: Yes.

JOHNSON: "After [Baby C] had died, I was still looking after JE. That would have been my focus."

Just to bring it back to mind, this particular shift, this is the shift where your colleague [Nurse B] suggested that she had to keep, in effect, removing you from the parents' room as [Baby C] was dying and just after he'd died. Do you remember that evidence?

LETBY: Yes.

JOHNSON: I'm going to suggest that your assertion in your defence statement that your recollection of events prior to [Baby C]'s collapse being vague is not true. What do you say?

LETBY: I don't agree with that.

JOHNSON: You don't. And I'm going to suggest that you enjoyed what happened and that's why you were in and out of the family room.

LETBY: No.

JOHNSON: You had been off on Thursday the 11th and Friday, 12 June, hadn't you?

LETBY: Yes.

JOHNSON: And you were back in on the night shift of Saturday the 13th?

LETBY: Yes.

JOHNSON: When you were being interviewed, did you remember [Baby C]?

LETBY: Yes.

JOHNSON: Why did you say that you didn't?

LETBY: I'm not sure what you mean. My police interview?

JOHNSON: Yes.

LETBY: I'm not sure what detail I had to start with, but I know I remembered him once I had more details. I'm not sure what I was told at the very beginning.

JOHNSON: We'll come to the detail. Is it your case that staffing levels contributed to [Baby C]'s death?

LETBY: No, I don't know how [Baby C] died.

JOHNSON: Is it your case that medical competence or incompetence contributed?

LETBY: No.

JOHNSON: Or that anybody made a mistake?

LETBY: No.

JOHNSON: Do you accept that [Baby C] progressed well having been born in good condition?

LETBY: I... I can only go on what other people's notes were, I'd not looked after [Baby C] before.

JOHNSON: But do you accept that the evidence is that [Baby C] was doing well?

LETBY: I think there had been issues with his abdomen and things, but...

JOHNSON: He wasn't perfectly healthy, that's why he was in a neonatal unit, but there were no causes for concern, were there?

LETBY: Not that I recall, no.

JOHNSON: No. I will just remind you of some of the context. The jury and you may remember that this is a case where one of the notes in the nursing notes was misfiled on a different baby's file and when [Baby C] had died, it was transferred from that file to [Baby C]'s file. It was a note made by Yvonne Griffiths; do you remember that?

LETBY: Yes.

JOHNSON: And just while we're on that subject, once you have entered -- we've seen in many cases, haven't we, that in the nursing notes the time that the file entry is opened is recorded by a computer?

LETBY: Yes.

JOHNSON: And the time that it is entered into the system is also recorded?

LETBY: Yes, the time it's completed, yes.

JOHNSON: Yes. Once a nursing note is completed, is it editable?

LETBY: No.

JOHNSON: And that perhaps is the reason why this particular note couldn't simply be transferred across, it had to be rewritten?

LETBY: Yes.

JOHNSON: Okay. Let's start with that because that gives us the position on the day shift of Friday, 12 June. It's tile 273, please, Mr Murphy.

If we can just go to that. These concern events at 18.30 on the day shift of Friday, 12 June. Just to remind us all, YG, Yvonne Griffiths, we can see that the note's actually entered after [Baby C] had died. At the time that -- we've got an eight-minute window in which these were typed, between 17.38 and 17.46, on 14 June.

We see that these are characterised or described as:

"On reflection notes from 12/6/15 for day shift, written in [redacted]'s notes for [Baby C]."

So presumably that was the family name of the child in whose notes this was entered erroneously; do you agree?

LETBY: Yes.

JOHNSON: So nursing notes printed and rewritten as follows and we see -- I think I said this was the day shift and it's the night shift, I beg your pardon:

"Assumed care of infant at 20.00 hours. Assistance with ventilation throughout the day..."

I think it was the day shift, actually. 20.00 hours may be wrong:

"By N CPAP in a pressure [and it gives the figures]. Nares [which we know are nostrils] red but not broken. DuoDERM applied. No apnoeas noted. Caffeine given as prescribed (inaudible) long line. [Baby C] unsettled at times, soothes with pacifier. Enjoyed kangaroo care with parents."

And then material about the X-ray of the long line and the fact that the NGT had been aspirated and 2ml of black-stained fluid obtained.

[Baby C] had been having -- what is kangaroo care, would you just remind us, please?

LETBY: It's when the baby takes its clothes off and is put against the mother or father's chest skin.

JOHNSON: Otherwise known as skin-to-skin?

LETBY: Yes.

JOHNSON: When a child is enjoying kangaroo care with parents, of course the child is not on any form of breathing support; is that right?

LETBY: No, we can do kangaroo care with breathing support, but they would be stable enough to be brought out.

JOHNSON: All right. But as a matter of fact, I'll suggest that [Baby C] had been off breathing support, and we'll come to some

documentation in a moment, but if we go backwards, please, Mr Murphy, although forwards in time, to T41.

This is Yvonne Griffiths -- sorry, Mel Taylor's nursing note and we see in the centre of the screen there to the left -- in the left-hand half:

"Remained nil by mouth. Tummy soft. Not distended. Dark bile obtained from OGT. Registrar Davies aware. To continue nil by mouth for now and monitor. Bowels not opened. Passed urine."

Right?

LETBY: Yes.

JOHNSON: So that's overnight, the 12th to the 13th. The day shift, in other words the shift immediately before the shift on which you were working and [Baby C] died, it was Joanne Williams who looked after him. I just want to deal with what happened on that day.

If we start, please, with the nursing at tile 116.

So we see there at the top JW for Joanne Williams.

Creation time, 16.19:

"Nursed on CPAP [pressures given]. Capillary blood gas at 9.00, very good. However, [Baby C] very unsettled and fractious. After discussion with Registrar Ogden, taken off CPAP whilst out having skin-to-skin with mum. Calmed down straightaway while on mum. Just required some facial oxygen while out. No signs of increased work of breathing..."

You read it all to yourself if you would, please.

Is that a positive picture?

LETBY: (Pause). Yes.

JOHNSON: Can we go to tile 67, please? This document gives us a ready reckoner for what sort of breathing support [Baby C] was on, doesn't it?

LETBY: Yes.

JOHNSON: Do we see from that document that he was on, on the 13th, so this is the day on which he died, that he was on CPAP up to 10 am, 10.00 hours?

LETBY: Yes.

JOHNSON: There was nothing at all, no breathing support other than, if we remember what was in the nursing note, the ambient oxygen --

LETBY: Yes.

JOHNSON: -- at 11 and 12?

LETBY: Yes.

JOHNSON: And he then went on to Optiflow -- so not CPAP, Optiflow -- at 13.00 hours?

LETBY: Yes.

JOHNSON: And that remained the position until the event that terminated with his or culminated with his death?

LETBY: Yes.

JOHNSON: If we go to tile 120 next, please, it's another note of Joanne Williams. It's the note at 18.00 on the 13th.

It may be the top of the next side, please. That's it:

"Capillary blood gas at 18.00 showed improvement from previous capillary blood gas. However, lactate risen to 2.5 millimoles. SHO informed. Remains stable on Optiflow. Blood pressure stable. Aspirates reduced this evening. [Query] commence enteral feeding tonight."

Do you remember that?

LETBY: Yes.

JOHNSON: And do you remember that it was Dr Gibbs who directed the commencement of enteral feeding?

LETBY: I can't remember who authorised it, but I know, yes, doctors had agreed he could start feeds, yes.

JOHNSON: And do you remember the evidence of Dr Marnerides, the pathologist, who said that the histology shows that [Baby C] did not have any bowel issue?

LETBY: Yes.

JOHNSON: Let's move on then to the shift on which you were present. It's tile 138, please, Mr Murphy, if you would.

Here we see the population list of the NNU, 12 children on the unit.

LETBY: I can just see the staff at the moment.

JOHNSON: It's my fault, I'm sorry. There it is at the bottom of the page. So we see the population list at the bottom of the page. We see that you had a child, JE.

LETBY: Yes.

JOHNSON: That's at number 3 there. As a matter of fact, JE was in nursery 3, wasn't he, do you remember?

LETBY: Yes.

JOHNSON: Sorry, I missed your answer. Do you remember that he was in nursery 3?

LETBY: Yes, sorry.

JOHNSON: And this, to bring it back, was -- he was the child with the breathing problems that [Nurse B] thought were worthy of close monitoring.

LETBY: Yes.

JOHNSON: Your other charge that evening was PE, who is number 10 in the list; do you see?

LETBY: Yes.

JOHNSON: And so far as nursery 1 was concerned, we heard evidence from several witnesses that there were two children in -- although we haven't got a population distribution chart, we heard from several witnesses that [Baby C] was in nursery 1.

LETBY: Yes.

JOHNSON: And so was another child who was being looked after by Melanie Taylor, who we see in the list at number 8, MRE.

LETBY: Yes.

JOHNSON: Do you recall there being any other children in nursery 1?

LETBY: No.

JOHNSON: So the evidence suggests, would you agree, that there were two children in nursery 1, one of whom was [Baby C], the other MRE?

LETBY: From this, yes.

JOHNSON: Yes. Well, not only from this but from all the evidence that we've heard in the trial --

LETBY: Yes.

JOHNSON: -- relating to this event, so from all your colleagues who gave evidence.

LETBY: That there were three babies in nursery 1?

JOHNSON: No, two babies in nursery 1: two in number 1 and you were looking after two in number 3.

LETBY: Okay. I'm not sure. If that's what they've said, so...

JOHNSON: I'm suggesting that's the evidence, and I'm sure I will be corrected if I'm wrong, and if I find out off my own bat I've made a mistake, I will correct it.

MR MYERS: I can't remember sitting here what it was but I couldn't remember it being put like that --

MR JUSTICE GOSS: It can be checked. No, no, I wasn't expecting you to.

MR MYERS: I know your Lordship wasn't. I couldn't remember it myself.

MR JUSTICE GOSS: Nor could I.

MR JOHNSON: There's a degree of preparation that goes in, but I'm not saying I'm incapable of making a mistake.

Anyway. First of all, I'm going to suggest to you that this was yet another shift on which you migrated from either nursery 3 or 4 back into nursery 1.

LETBY: Yes, in response to [Baby C]'s care needs.

JOHNSON: No, before [Baby C] collapsed.

LETBY: I don't have a recollection of that.

JOHNSON: Do you remember [Nurse B] told us she was the shift leader on this shift?

LETBY: Yes.

JOHNSON: And do you remember her telling us that she had to tell you to look after him rather than getting yourself involved with other children?

LETBY: Yes.

JOHNSON: Is that true?

LETBY: No. I don't recall that conversation.

JOHNSON: Are you saying you reject it or simply you don't remember?

LETBY: I do not remember a conversation like that happening.

JOHNSON: I'm going to suggest that you were very unhappy with the arrangements that she had made, which dictated where you were working. Do you agree that you were unhappy at being in nursery 3?

LETBY: Yes, but that wasn't down to [Nurse B], it was down to the shift leader on the previous shift.

JOHNSON: Right.

MR JUSTICE GOSS: But you were unhappy about being in 3?

LETBY: Yes, I expressed that I would have liked to go into nursery 1, yes.

MR JUSTICE GOSS: You say that was the decision of the shift leader?

LETBY: Prior to this --

MR JUSTICE GOSS: Prior -- yes, for the previous shift?

LETBY: So it wasn't [Nurse B], it was whoever was on before that as the shift leader.

MR JOHNSON: Yes, we have heard a bit of evidence about that from various people, but as a matter of practice, does the leader of the shift on which the nurses are actually working have the capacity to redistribute labour?

LETBY: Yes.

JOHNSON: So if you had asked [Nurse B], she would have had the power to redistribute, to put you somewhere else if she thought that was appropriate?

LETBY: Yes.

JOHNSON: As a matter of fact, do you agree that she thought JE needed your attention?

LETBY: He did need attention, yes, and I gave him that attention.

JOHNSON: Let's look at how unhappy you were because I'm suggesting this is precisely how you were. Starting at tile 146, please. This is your friend, Jennifer Jones-Key, asking if you're all right.

147. You read to us what you replied.

LETBY: "I just keep thinking about Monday, feel like I need to be 1 to overcome it but [Nurse B] said no."

JOHNSON: Monday was [Baby A]; is that right?

LETBY: Yes.

JOHNSON: 148 is Jennifer Jones-Key:

"I agree with her, don't think it will help. Need a break from full-on ITU. You have to let it go or it will eat you up. I know not easy and will take time."

Then 149. Your message, please.

LETBY: "Not the vented baby necessarily, I just feel I need to be in 1 to get the image out of my head. Mel said the same and [Nurse B] let her go. Being in 3 is eating me up. All I can see is him in 1."

JOHNSON: The vented baby we know was not [Baby C], don't we?

LETBY: Yes.

JOHNSON: This was the other baby in nursery 1, who was ME or MAE as he appears in the list; is that right?

LETBY: Yes, on the list yes.

JOHNSON: So you wanted to look after [Baby C], didn't you, if there were two babies in nursery 1?

LETBY: I don't think I'm referring to specifically [Baby C].

I can't...

JOHNSON: There's a field of two available for care in nursery 1.

You want to be in nursery 1 and you're saying, well, okay, I won't have the vented baby. Who does that leave?

LETBY: Okay, if that's true and there were only two babies in that nursery then, yes, it would fall to [Baby C], yes.

JOHNSON: Is that the truth of it, that you wanted to look after [Baby C]?

LETBY: No. Not [Baby C] specifically, no.

JOHNSON: 150 next, please. If you'd read your own messages I'd be grateful.

LETBY: "It probably sounds odd but it's how I feel."

JOHNSON: And 151. Jennifer Jones-Key replied:

"Well, it's up to you but I don't think it's going to help. It sounds very odd and I would be the complete opposite. Can understand [Nurse B], she's trying to look after you all."

And then you replied?

LETBY: "Well that's how I feel. From when I've experienced it at Women's I've needed to go straight back and have a sick baby otherwise the image of the one you lost never goes. Why send Mel in if she's trying to look after us? She was in bits over it."

JOHNSON: That's a reference to the fact that Melanie Taylor was looking after the other -- the vented baby in nursery 1, isn't it?

LETBY: Yes.

JOHNSON: And 153, please:

LETBY: "Don't expect people to understand but I know how I feel and how I have dealt with it before. I have voiced that so can't do any more, but people should respect that".

JOHNSON: You wanted to get your own way, didn't you?

LETBY: No, I expressed a preference to go to nursery 1, as did Mel.

JOHNSON: "And people should respect that", what does that mean in this context?

LETBY: People should respect what I'm asking and saying that that's how I potentially deal with something.

JOHNSON: So you should get your own way?

LETBY: No, it's not about having your own way, it's about how you deal with things. Mel and I had both asked to go into nursery 1.

JOHNSON: 160. Jennifer Jones-Key replies:

"I think they do respect it, but also trying to help you. Why don't you go in 1 for a bit?"

What did you take that to mean?

LETBY: That the staff there are trying to help me.

JOHNSON: Well, Jennifer Jones-Key isn't working, is she?

LETBY: No.

JOHNSON: Right. Okay. So what did you understand her to be saying to you by:

"Why don't you go in 1 for a bit"?

LETBY: Spend some time in nursery 1.

JOHNSON: Yes. And your response?

LETBY: "Yeah, I've done a couple of meds in 1, I'll be fine."

JOHNSON: What did that mean?

LETBY: That I've checked or drawn up some medications in nursery 1.

JOHNSON: 162:

"It didn't sound like you would be. Sorry, was eating my tea."

I assume that's a reference to "you would be fine", was it?

LETBY: Yeah, I think so, yes.

JOHNSON: And then you responded and there's something, part of a message relating to somebody that has nothing to do with this. Then when did you say?

LETBY: "Forget I said anything, I'll be fine. It's part of the job. Just don't feel like there is much team spirit tonight."

JOHNSON: Did you feel that you weren't getting your own way?

LETBY: It wasn't about me having my own way.

JOHNSON: She replied:

"I'm not going to forget, but just think you're way too hard on yourself. It is part of the job but the worst part. I do believe it makes us stronger people."

So that takes us to 164. The conversation continued at 167. So we are now at 22.53. Could you read this one that you sent to Jennifer Jones-Key?

LETBY: "Unfortunately, I've seen my fair share at the Women's but you are supported differently and here it's like people want to tell you how to think/feel. Anyway, onwards and upwards. Just shame I'm on with Mel and [Nurse B]. Sophie in 1 so haven't got her to talk to either."

JOHNSON: 167 -- of course, Sophie was looking after [Baby C], wasn't she?

LETBY: Yes.

JOHNSON: Then Jennifer Jones-Key responded:

"Work is work. A lot of girls say Women's don't support and tell them to get on with it. I think they don't mean to tell you though and were overcaring sometimes. Yeah, that's not good but you got Liz."

Who is Liz in that context?

LETBY: Elizabeth Marshall, the band 4 that was on.

JOHNSON: The nursery nurse?

LETBY: Yes.

JOHNSON: Okay. We then resume the conversation at 174. Could you read your message, please?

LETBY: "Women's can be awful but I learnt the hard way that you have to speak up to get support. I lost a baby one day and few hours later was given another dying baby just born in the same cot space. Girls there said it was important to overcome the image. It was awful but by end of the day I realised they were right. It's just different here."

JOHNSON: And then, a minute later?

LETBY: "Anyway, forget it. I can only talk about it properly with those who knew him and Mel not interested, so I'll overcome it myself. You get some sleep."

JOHNSON: Were you upset?

LETBY: Yes.

JOHNSON: Were you upset with Mel?

LETBY: No, I was upset just generally that I didn't feel my feelings were being considered.

JOHNSON: By who?

LETBY: By [Nurse B] and Mel.

JOHNSON: And what did you expect Mel to do for you?

LETBY: That Mel and I had both been present with [Baby A] and that we could have supported one another with that.

JOHNSON: So you wanted her to talk to you about [Baby A]'s death, is that it?

LETBY: I wanted her to be there to support me, yes. And she had wanted to go in nursery 1 for the same reason and she had been put in there.

JOHNSON: According to what you said this morning, Mel was responsible or potentially responsible for [Baby A]'s death?

LETBY: Potentially, yes.

JOHNSON: And you wanted her to talk to you about it?

LETBY: No, I think what I'm suggesting here is the only way you overcome these things is by talking to colleagues that were also present.

JOHNSON: Shared experience?

LETBY: Yes.

JOHNSON: Is that what you enjoyed?

LETBY: Can you rephrase that?

JOHNSON: Is that what you enjoyed?

LETBY: What, talking to colleagues? I don't...

JOHNSON: No, sharing experience of babies collapsing and dying.

LETBY: No, we share experiences of very many reasons, not just that.

JOHNSON: Well, Jennifer Jones-Key responded:

"That's a bit mean, isn't it? Don't have to know him to understand. We've all been there. Yep, off to bed now."

You replied at 178?

LETBY: "I don't mean it like that, just that only those who saw him know what image I have in my head."

JOHNSON: And then the next one, please.

LETBY: "Forget it. I'm obviously making more of it than I should."

JOHNSON: And then you said, "Sleep well", at 180. A couple of times there you've said "forget it"; were you frustrated?

LETBY: Yes, I felt I wasn't getting anywhere with the conversation and I just wanted to leave it at that point.

JOHNSON: This is the conversation with Jennifer Jones-Key?

LETBY: Yes.

JOHNSON: What were you expecting her to say that she was not saying? What were you wanting from --

LETBY: That she could maybe have been a little bit more understanding about what I was saying and how I was feeling.

JOHNSON: Did you want attention?

LETBY: I don't think attention's the right word, I think it's just recognition from a friend to acknowledge how I was feeling.

JOHNSON: Because you've been through such a dreadful experience, witnessing the death of [Baby A], you wanted people to show you what?

LETBY: I don't think I wanted them to show me anything. She is a friend, I wanted to express how I felt to her and I didn't feel she fully understood the context in which I was saying things, so I wanted to leave the conversation.

JOHNSON: Sophie Ellis, Melanie Taylor and [Nurse B] were all asked questions on your behalf on the basis that Sophie Ellis was insufficiently qualified to have been looking after [Baby C].

LETBY: Yes.

JOHNSON: Do you remember that?

LETBY: Yes.

JOHNSON: Is that your position?

LETBY: Yes.

JOHNSON: So does it come to this then: you wanted to be in nursery 1?

LETBY: Yes.

JOHNSON: Your wish was being frustrated by management?

LETBY: By the shift leader, not by management, but the shift leader, yes.

JOHNSON: And in your view, the person who had what you wanted, in other words to be looking after the non-vented baby in nursery 1, wasn't sufficiently qualified for the job?

LETBY: No, Sophie wasn't, I don't think, in the correct position to care for [Baby C].

JOHNSON: Why's that?

LETBY: She was recently qualified. She did not have the experience or the skills.

JOHNSON: Which particular skill was she deficient in?

LETBY: She didn't have experience of premature babies, small babies like [Baby C].

JOHNSON: But which -- you see, one of the questions I asked you several minutes ago now was whether there was some sort of deficiency in care that led to [Baby C]'s death. So I'd like you to explain to us why it is Sophie was insufficiently qualified to being doing the job she was doing.

LETBY: I'm not saying Sophie caused anything with [Baby C], I'm just saying she was the least experienced member of staff on that shift, she was in nursery 1, the intensive care room, with another ventilated baby and she had very little experience with small premature babies like [Baby C] that we need to keep a close observation of.

JOHNSON: So she had something you wanted?

LETBY: No.

JOHNSON: The care of [Baby C], wasn't that what you wanted?

LETBY: No, not specifically [Baby C], no.

JOHNSON: Can we go to [Initials of Baby C] 13, please, which is the second of the [Baby C] interviews. Here you can see there's a long question towards the top of the page.

LETBY: Which page, sorry?

JOHNSON: [Initials of Baby C] 13. It's the second interview.

LETBY: Yes.

JOHNSON: My fault. So there's a long question that runs to about 10 lines in the top third of the page; do you see that?

LETBY: Yes.

JOHNSON: What the police are asking about is some of the text messages that we've referred to.

LETBY: Yes.

JOHNSON: They read them to you. They ask you whether you recall the conversation. You say no. Then the question:

"Question: Can you tell me where it -- where you were when it took place?

"Answer: I don't remember it. What do the messages relate to?"

What did you say then?

LETBY: "I don't remember".

JOHNSON: Was that the truth?

LETBY: Yes, because at this point that message had just been read out to me without any context.

JOHNSON: Isn't the context fairly obvious from the messages themselves?

LETBY: Not when I was being interviewed about multiple babies at multiple times and this was just read out with no context. I did not immediately link that to that shift.

JOHNSON: All right. Let's see what the context was if we go back to page 11, please. Do you see there at 29.10, they're talking about a statement that's been taken from Sophie Ellis --

LETBY: Yes.

JOHNSON: -- who said she was out of the room when [Baby C]'s alarm sounded; is that right?

LETBY: Yes.

JOHNSON: That you, Lucy Letby, had your own designated baby in nursery 3?

LETBY: Yes.

JOHNSON: Sophie Ellis says that when she went into nursery 1, you were standing over or standing next to [Baby C]'s cot?

LETBY: Yes.

JOHNSON: "Question: Sophie Ellis says that you said, 'He's just dropped his heart rate and saturations', or something similar. Do you recall saying that to Sophie Ellis?"

Then over the page:

"Question: You were placed into nursery 3 during this shift, had your own designated baby to look after [and you said you didn't remember]. Have you any explanation as to why you were in nursery 1 when the alarms went off?"

And you said you didn't remember. You mention that you might have been doing checks in nursery 1; is that right?

LETBY: Yes.

JOHNSON: You were asked whether you'd been treating [Baby C] and you say not that you remember, you couldn't remember specifically why you were there. Do you see that?

LETBY: Yes.

JOHNSON: They then, having laid out all that to you, turn to the telephone conversation that we've just dealt with.

LETBY: Yes.

JOHNSON: Was that not sufficient context for you to remember what it was all about?

LETBY: No.

JOHNSON: Let's see what was happening at about this time in the neonatal review, please. So if we can go to that document in the white bundle, please.

Here we see quite a lot of blue material or blue typeface, I should say, which reflects what you were doing; is that right?

LETBY: Yes.

JOHNSON: So at line 2 at 20.00, line 6, 21.00, 17 and 20, 21.10 and 21.30, you're dealing with the children JE and PE.

Is that right? Then these are simply making entries on charts, observations, aren't they?

LETBY: Yes.

JOHNSON: No medications there, are there?

LETBY: No.

JOHNSON: Can you find us a medication that you were responsible for on this page?

LETBY: The one at line 38 for HM.

JOHNSON: HM was not one of the babies in nursery 1?

LETBY: No, it was my baby in nursery 3.

JOHNSON: Yes. Any other medications there (inaudible) at 45?

LETBY: Yes.

JOHNSON: Well, actually --

LETBY: The medication for NM.

JOHNSON: Yes, we've got JE at 20.00 hours in nursery 3; is that right?

LETBY: Yes.

JOHNSON: We've got PE and HM and EB at 37 to 41?

LETBY: Yes.

JOHNSON: 20.00 hours to 22.50.

LETBY: Yes.

JOHNSON: We've got NM at lines 45 and 46 at 22.21?

LETBY: Yes.

JOHNSON: None of those babies were in nursery 1, were they?

LETBY: No, but the medications are drawn up in nursery 1.

JOHNSON: Well, I know that's what you say, but context, as you say, is everything.

If we go back -- I'll check I've not missed something. No.

If we go back to line 161 -- sorry, tile 161:

"I've done a couple of meds in 1, I'll be fine."

That's a message you sent at 22.34?

LETBY: Yes.

JOHNSON: You hadn't actually given any meds to any children in nursery 1, had you?

LETBY: No.

JOHNSON: As you say, context being important, when we looked at that conversation, what you wanted was to be treating one of the children in nursery 1?

LETBY: Yes, at the start of my shift, yes.

JOHNSON: And that's what you were suggesting you had been doing to your friend, Jennifer Jones-Key, isn't it?

LETBY: Clinical practice is IV medication is always drawn up in nursery 1. That is where the medications are all stored and kept, oral and IV. It could not be made up in a special care nursery.

JOHNSON: Are you covering yourself?

LETBY: No, it's standard practice.

JOHNSON: At 23.00 hours, we know that Sophie Ellis gave [Initials of Baby C], [Baby C], his first feed and did his observations, don't we?

LETBY: Yes.

JOHNSON: Do you accept her evidence that she aspirated his stomach and obtained a little bit of green bile?

LETBY: Yes.

JOHNSON: Do you accept that if that's what she did, there wasn't any air in his stomach?

LETBY: No, if she's documented correctly, then yes.

JOHNSON: So no air in [Baby C]'s stomach at 23.00 hours?

LETBY: No, according to Nurse Ellis, yes.

JOHNSON: Have you any reason to doubt what she says?

LETBY: No.

JOHNSON: And then Nurse Ellis said that she did some meds for other children. Do you remember her saying that?

LETBY: Not specifically now, no.

JOHNSON: Rather than relying on our memories, perhaps we can rely on the neonatal review at lines 66 to 70. These are medications for MRE, who is, as a matter of fact, Melanie Taylor's other child or her charge in nursery 1.

LETBY: Yes.

JOHNSON: She then told us that she left nursery 1; do you remember that?

LETBY: Yes.

JOHNSON: And that was the opportunity that you took to sabotage [Baby C], wasn't it?

LETBY: No.

JOHNSON: If we go to your interview, please -- this is the second of your interviews, please. If we start at [Initials of Baby C] 11 to give us the context. This is the interview that we looked at a short time ago; all right?

LETBY: Yes.

JOHNSON: It's dated 11 June 2019 and we have reviewed the first two and a half pages, which precede the text conversation that I was asking you about, about 10 or so minutes ago.

LETBY: Yes.

JOHNSON: If we move on, we can see that the conversation -- sorry, the questions at the bottom of page 13 included references to more of the texts that I asked you about.

LETBY: Yes.

JOHNSON: As it continued over on to page 14 --

LETBY: Yes.

JOHNSON: -- and 15 --

LETBY: Yes.

JOHNSON: -- which is where you say, after the officer reading out quite a long series of texts, which are at tiles 175 through to 178, that you were frustrated with Jennifer Jones-Key, with her response; is that right?

LETBY: Disappointed, more, I would say, but yes.

JOHNSON: Well, I'm using your word.

LETBY: Okay, yes.

JOHNSON: Frustrated. Then do you see a bit further down the page it says:

"Okay. 23.09, Lucy, is when you sent that final message to Jen on that shift. Where were you when you sent that message?"

And you say that you don't remember.

You say that in one of the messages at 21.55 -- they repeat back to you your message:

"Being in 3 is eating me up."

Which we have at tile 149. Do you see that?

LETBY: Yes, because I was unsure whether I was on shift that day.

JOHNSON: Yes. Well, are you suggesting that the death of [Baby C] wasn't very memorable?

LETBY: I don't think that's a fair point. I think at this point I was being interviewed by (sic) so many babies on various dates, I could not recall every baby that I'd looked after or that this was specifically [Baby C] at that time.

JOHNSON: Well, I'm not going to re-plough the field we ploughed earlier to give you the context.

[Initials of Baby C] 16, please. The police conclude their review, as I reviewed with you, of that series of text messages.

Do you see there?

LETBY: Yes.

JOHNSON: They say about seven lines down:

"So you sent that file text to Jen at 23.09 and [Baby C] collapsed at 23.15?"

LETBY: Yes.

JOHNSON: "So 6 minutes after you've sent that message, [Baby C] has collapsed?"

LETBY: Yes.

JOHNSON: "What are your thoughts on that?"

And your response?

LETBY: "I don't recall where I was at the time that I send that text. I might have been at the nurses' station and then have gone into nursery 1 to do something else [as read]."

JOHNSON: Yes. They're not telling you were in nursery 1, are they, at that point?

LETBY: Yes, I believe beforehand, Sophie had suggested that I was there --

JOHNSON: Oh yes, right at the beginning of the interview.

LETBY: Yes.

JOHNSON: You are here accepting, aren't you, that you were in nursery 1 when you finished that series of texts with your friend, Jennifer Jones-Key, and you were feeling frustrated?

LETBY: No, I can't confirm that I sent that message in nursery 1, no.

JOHNSON: No, not sent the message, you are confirming that you went -- having sent the message, you went into nursery 1?

LETBY: No, I'm not saying that, I don't think. I'm saying I don't recall where I was.

JOHNSON: Yes, you say:

"I don't recall where I was at the time that I sent that text."

LETBY: Yes.

JOHNSON: "I might have been at the nurses' station and then I've gone into nursery 1 to do something else."

LETBY: Yes, because just previous to that it was stated to me that Sophie Ellis had seen me at the cot side and I took that to be accurate.

JOHNSON: It is accurate, isn't it?

LETBY: It was on her word at that time.

JOHNSON: You were quite happy to accept it, were you?

LETBY: I haven't accepted it, I've said I don't recall.

JOHNSON: The death of [Baby C] was very, very memorable, wasn't it?

LETBY: Yes.

MR JOHNSON: My Lord, that may be a convenient point.

MR JUSTICE GOSS: Right, members of the jury. That's it for today. 10.30 tomorrow morning, please.

(4.14 pm)

(The court adjourned until 10.30 am on Friday, 19 May 2023)